Anxiety, Trauma- and Stressor-Related, and Obsessive-Compulsive and Related Disorders:

When Fear Takes Control



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Learning Outcomes:

At the end of this session, students should be able to:

1. **Differentiate** anxiety, fear, panic, obsessions, compulsions, and trauma-related reactions.
2. **Explain** the major clinical features and integrative causes of selected anxiety, trauma-related, and obsessiwe-compulsive disorders.
3. **Apply** diagnostic concepts to brief clinical situations while identifying appropriate psychological interventions.



Topic Outline:

01 **Understanding Anxiety**

- Anxiety, fear, and panic
- Causes and maintaining factors
- Comorbidity and suicide

02 **Anxiety Disorders**

- Generalized Anxiety Disorder
- Panic Disorder & Agoraphobia
- Specific Phobia
- Social Anxiety Disorder

03 **Trauma & Stress**

- Posttraumatic Stress Disorder

04 **OCD & Related Disorders**

- Obsessive-Compulsive Disorder
- Body Dysmorphic Disorder
- Hoarding Disorder
- Trichotillomania & Excoriation

05 **Clinical Application**

- Case analysis
- Treatment matching
- Key takeaways



ICEBREAKER

FEAR OR ANXIETY?

Read each situation:

A. You hear footsteps behind you while walking alone at night.

B. You keep thinking that something terrible will happen tomorrow—even though nothing specific is happening.

C. Your heart suddenly races, you feel dizzy, and you think you are about to die.



QUESTION

Are these the same psychological experience? Why or why not?

Anxiety Is Not Automatically a Disorder

Anxiety can be:

Adaptive
→ prepares us for possible danger

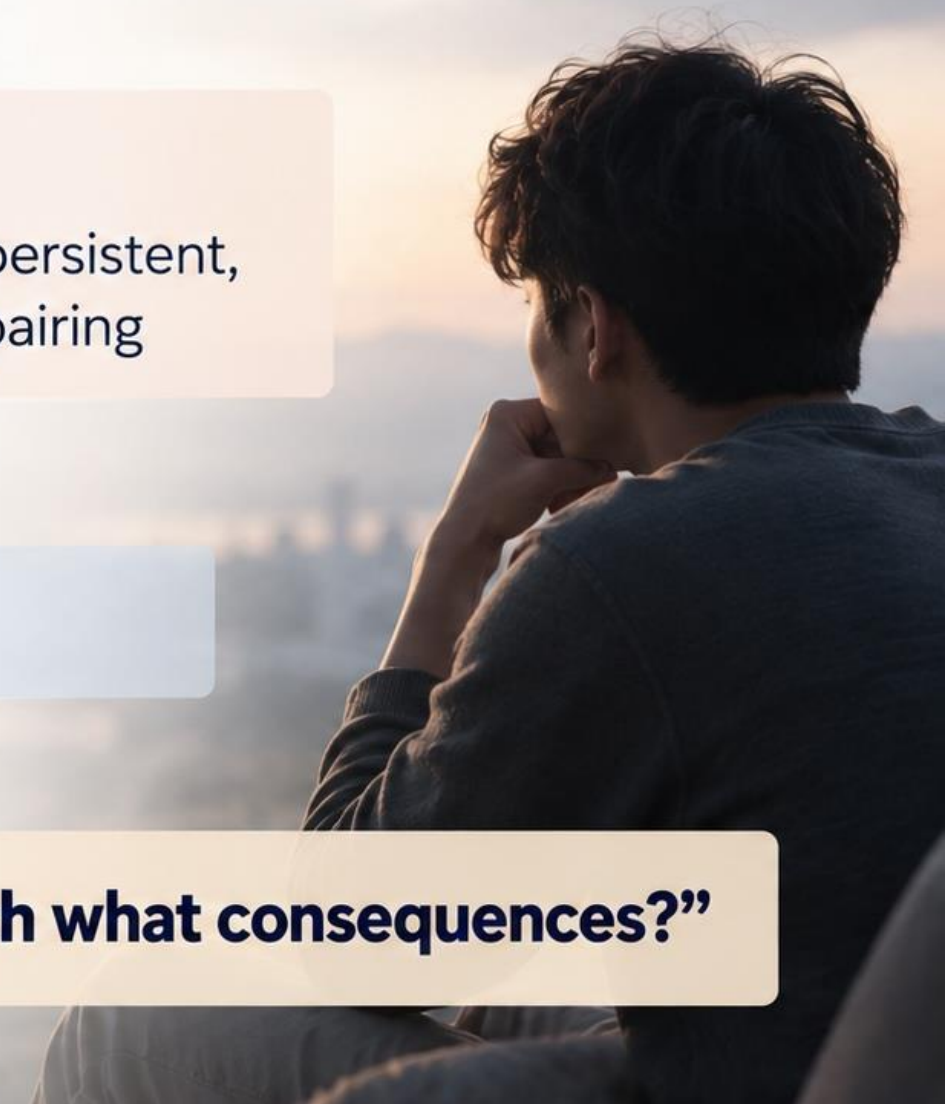
Maladaptive
→ becomes excessive, persistent, uncontrollable, or impairing

The key question is not:

“Does the person feel anxious?”

Instead:

“How much, how often, in what context, and with what consequences?”



Anxiety, Fear, and Panic

THREE RELATED BUT DIFFERENT EXPERIENCES



FEAR

Response to a **present or imminent threat**

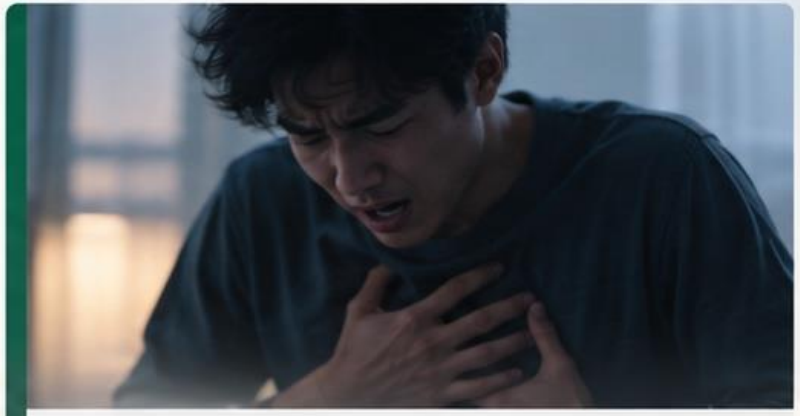
"Something dangerous is happening NOW."



ANXIETY

Future-oriented response to **possible threat**

"Something bad MIGHT happen."



PANIC

Sudden surge of intense fear or discomfort

Common experiences:

- racing heart
- shortness of breath
- dizziness
- trembling
- chest discomfort
- fear of losing control or dying

WHEN ANXIETY BECOMES A DISORDER

FROM NORMAL ANXIETY → PSYCHOPATHOLOGY

Consider:

INTENSITY
How strong is the response?

DURATION
How long does it persist?

FREQUENCY
How often does it occur?

CONTROL
Can the person regulate or stop it?

IMPAIRMENT
Does it interfere with life?

AVOIDANCE
Does the person increasingly avoid situations?



THE INTEGRATIVE MODEL

Anxiety disorders rarely have a single cause.

Biological

- genetic vulnerability
- temperament
- neurobiological processes

Psychological

- learning
- conditioning
- cognitive biases
- perceived lack of control

Social / Environmental

- family experiences
- stressful events
- social context
- cultural factors

Maintaining Factors

- avoidance
- safety behaviors
- negative reinforcement
- catastrophic interpretations



THE VICIOUS CYCLE

TRIGGER

↓

THREAT INTERPRETATION

↓

ANXIETY / FEAR

↓

AVOIDANCE OR SAFETY BEHAVIOR

↓

SHORT-TERM RELIEF

↓

FEAR IS NEVER DISCONFIRMED

↓

ANXIETY IS MAINTAINED



DISORDERS RARELY TRAVEL ALONE

People with anxiety-related disorders may also experience:

- depressive disorders
- other anxiety disorders
- substance-related problems
- physical health conditions
- other psychological difficulties



WHY DOES THIS MATTER?

Comorbidity can affect:

Assessment → **Diagnosis** → **Treatment** → **Prognosis**

SUICIDE AND ANXIETY-RELATED DISORDERS

A CLINICAL SAFETY ISSUE

Severe psychological distress can coexist with:

- hopelessness
- depression
- substance misuse
- functional impairment
- suicidal thoughts or behavior

Clinical principle:
"Suicide risk should be assessed directly when clinically indicated."

DO NOT ASSUME:
"Anxiety disorder = low suicide risk."



ANXIETY-RELATED DISORDERS

ANXIETY DISORDERS

Four major disorders emphasized today:

GENERALIZED ANXIETY DISORDER
“What if something goes wrong?”

PANIC DISORDER / AGORAPHOBIA
“What if I panic—or can’t escape?”

SPECIFIC PHOBIA
“I cannot face THAT.”

SOCIAL ANXIETY DISORDER
“What if people judge me?”



GENERALIZED ANXIETY DISORDER

Core pattern:

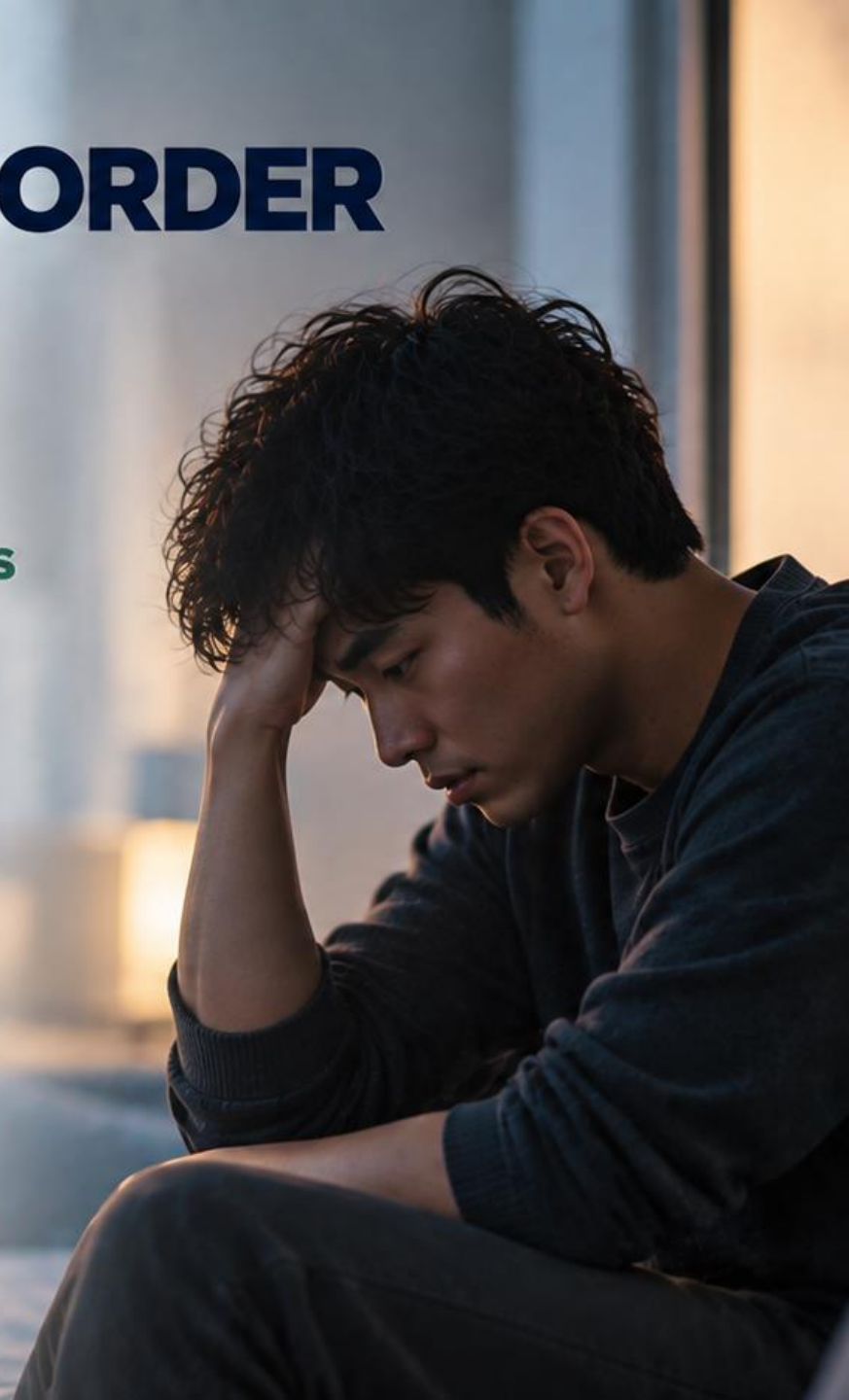
Excessive + difficult-to-control worry

Worry commonly involves:

- work/school
- health
- finances
- family
- everyday responsibilities
- future events

Associated symptoms may include:

- restlessness
- fatigue
- difficulty concentrating
- irritability
- muscle tension
- sleep disturbance



CENTRAL QUESTION

“Can the person control the worry?”

GAD: WHY DOES WORRY PERSIST?

WORRY CAN BECOME A COPING STRATEGY

A person may believe:

> *"If I worry enough, I'll be prepared."*

Worry may provide:

Temporary sense of control

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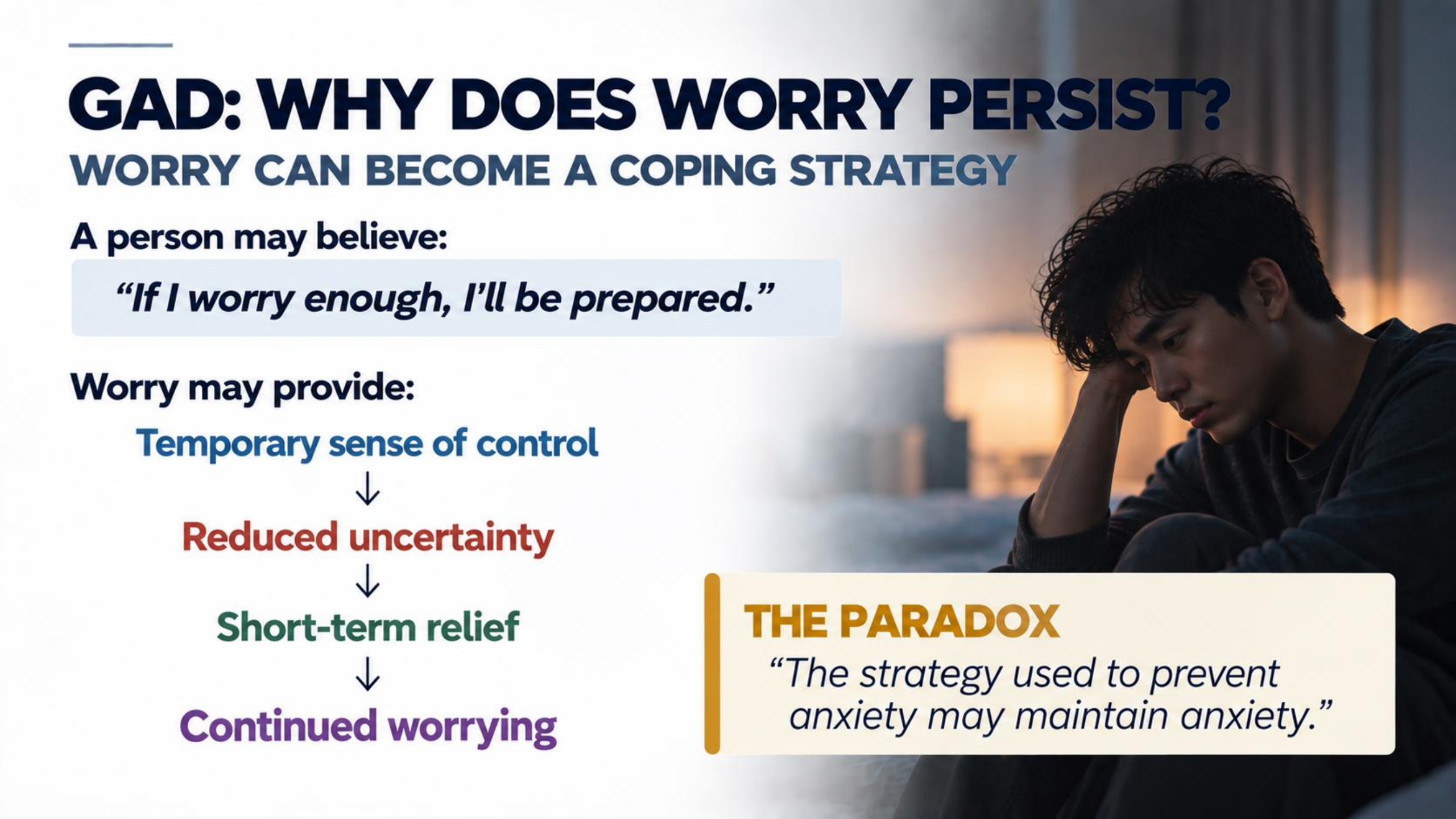
Reduced uncertainty

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Short-term relief

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Continued worrying



THE PARADOX

"The strategy used to prevent anxiety may maintain anxiety."

Generalized Anxiety Disorder

Diagnostic Criteria • F41.1

Diagnostic criteria

A. **Excessive anxiety and worry** (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it **difficult to control the worry**.

C. The anxiety and worry are associated with **three (or more) of the following six symptoms** (with at least some symptoms present more days than not for the past 6 months).

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.
2. Being easily fatigued.
3. Difficulty concentrating or mind going blank.
4. Irritability.
5. Muscle tension.
6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

Clinical significance and exclusions

D. The anxiety, worry, or physical symptoms cause **clinically significant distress or impairment** in social, occupational, or other important areas of functioning.

E. The disturbance is **not attributable to the physiological effects of a substance** (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is **not better explained by another mental disorder**.

Examples include anxiety or worry about having panic attacks in panic disorder; negative evaluation in social anxiety disorder; contamination or other obsessions in obsessive-compulsive disorder; separation from attachment figures in separation anxiety disorder; reminders of traumatic events in posttraumatic stress disorder; gaining weight in anorexia nervosa; physical complaints in somatic symptom disorder; perceived appearance flaws in body dysmorphic disorder; having a serious illness in illness anxiety disorder; or the content of delusional beliefs in schizophrenia or delusional disorder.

GENERALIZED ANXIETY DISORDER (GAD)

STATISTICS, CAUSES, AND TREATMENT

Statistics

- **2.7%** past-year prevalence among U.S. adults
- **5.7%** lifetime prevalence
- More common among **females**
- Often associated with **significant functional impairment**

Causes

- **Genetic/biological vulnerability**
- **Chronic stress or trauma**
- **Environmental factors**
- **Cognitive factors:** intolerance of uncertainty, threat overestimation, catastrophic thinking

Treatment

- **CBT** — major evidence-based psychological treatment
- **ACT** — acceptance, mindfulness, values-based action
- **Medication:** SSRIs/SNRIs; other options may include buspirone
- **Combined psychotherapy + medication** may be appropriate for some patients



PANIC DISORDER

PANIC ATTACK

A sudden surge of intense fear/discomfort accompanied by physical and cognitive symptoms.

PANIC DISORDER INVOLVES:

- Recurrent unexpected panic attacks
- Persistent concern or behavioral change related to future attacks

EXAMPLES:

- monitoring bodily sensations
- avoiding situations
- seeking reassurance
- changing routines



THE PANIC CYCLE

Bodily sensation

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“I might be in danger.”

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Catastrophic interpretation

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More physiological arousal

↓

More frightening sensations

↓

“I am losing control!”

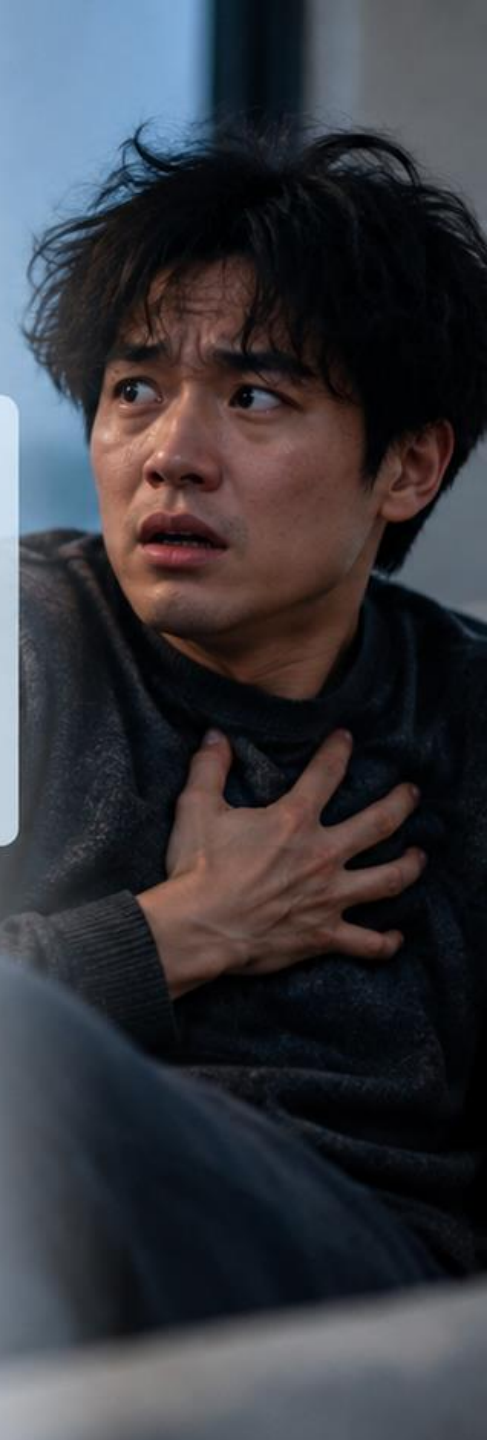
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PANIC ESCALATES



Panic Disorder

DSM-5-TR Diagnostic Criteria (F41.0)



A. **Recurrent unexpected panic attacks.** A panic attack is an abrupt surge of intense fear or intense discomfort that reaches a peak within minutes, and during which time four (or more) of the following symptoms occur.
Note: The abrupt surge can occur from a calm state or an anxious state.

1. Palpitations, pounding heart, or accelerated heart rate.
2. Sweating.
3. Trembling or shaking.
4. Sensations of shortness of breath or smothering.
5. Feelings of choking.
6. Chest pain or discomfort.
7. Nausea or abdominal distress.
8. Feeling dizzy, unsteady, light-headed, or faint.
9. Chills or heat sensations.
10. Paresthesias (numbness or tingling sensations).
11. Derealization (feelings of unreality) or depersonalization (being detached from oneself).
12. Fear of losing control or "going crazy."
13. Fear of dying.

Note: Culture-specific symptoms (e.g., tinnitus, neck soreness, headache, uncontrollable screaming or crying) may be seen. Such symptoms should not count as one of the four required symptoms.

B. **At least one of the attacks has been followed by 1 month (or more) of one or both of the following:**

1. Persistent concern or worry about additional panic attacks or their consequences (e.g., losing control, having a heart attack, "going crazy").
2. A significant maladaptive change in behavior related to the attacks (e.g., behaviors designed to avoid having panic attacks, such as avoidance of exercise or unfamiliar situations).

C. The disturbance is **not attributable to the physiological effects of a substance** (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism, cardiopulmonary disorders).

D. The disturbance is **not better explained by another mental disorder** (e.g., the panic attacks do not occur only in response to feared social situations, as in social anxiety disorder; in response to circumscribed phobic objects or situations, as in specific phobia; in response to obsessions, as in obsessive-compulsive disorder; in response to reminders of traumatic events, as in posttraumatic stress disorder; or in response to separation from attachment figures, as in separation anxiety disorder).

PANIC DISORDER

STATISTICS, CAUSES/RISK FACTORS, AND TREATMENT

Statistics

- Lifetime prevalence: approximately **4.7%** in U.S. adults.
- More common in **females** than males.
- Often begins during **late adolescence or early adulthood**.

Causes / Risk Factors

- Genetic vulnerability
- Heightened sensitivity to bodily sensations
- Neurobiological factors
- Stress and traumatic experiences
- Catastrophic interpretation of physical sensations

Treatment

- **CBT**, particularly **interoceptive exposure**.
- **SSRIs/SNRIs**.
- Benzodiazepines may provide rapid relief but have **dependence and tolerance risks**.
- **Combination of psychotherapy and medication** may be used when appropriate.



AGORAPHOBIA

Central concern:

Fear of situations where escape may be difficult or help may be unavailable if panic-like or other incapacitating symptoms occur.

Possible situations:

- public transportation
- open spaces
- enclosed spaces
- crowds
- being outside alone
- being far from home

KEY IDEA

The problem is not simply “fear of open spaces.”



Agoraphobia

DSM-5-TR Diagnostic Criteria (F40.00)

A. Marked fear or anxiety about **two (or more) of the following five situations:**

1. Using public transportation (e.g., automobiles, buses, trains, ships, planes).
2. Being in open spaces (e.g., parking lots, marketplaces, bridges).
3. Being in enclosed places (e.g., shops, theaters, cinemas).
4. Standing in line or being in a crowd.
5. Being outside of the home alone.

B. The individual fears or avoids these situations because of thoughts that **escape might be difficult** or **help might not be available** in the event of developing panic-like **symptoms** or other incapacitating or embarrassing symptoms (e.g., fear of falling in the elderly; fear of incontinence).

C. The agoraphobic situations **almost always provoke fear or anxiety.**

D. The agoraphobic situations are **actively avoided**, require the **presence of a companion**, or are endured with intense fear or anxiety.

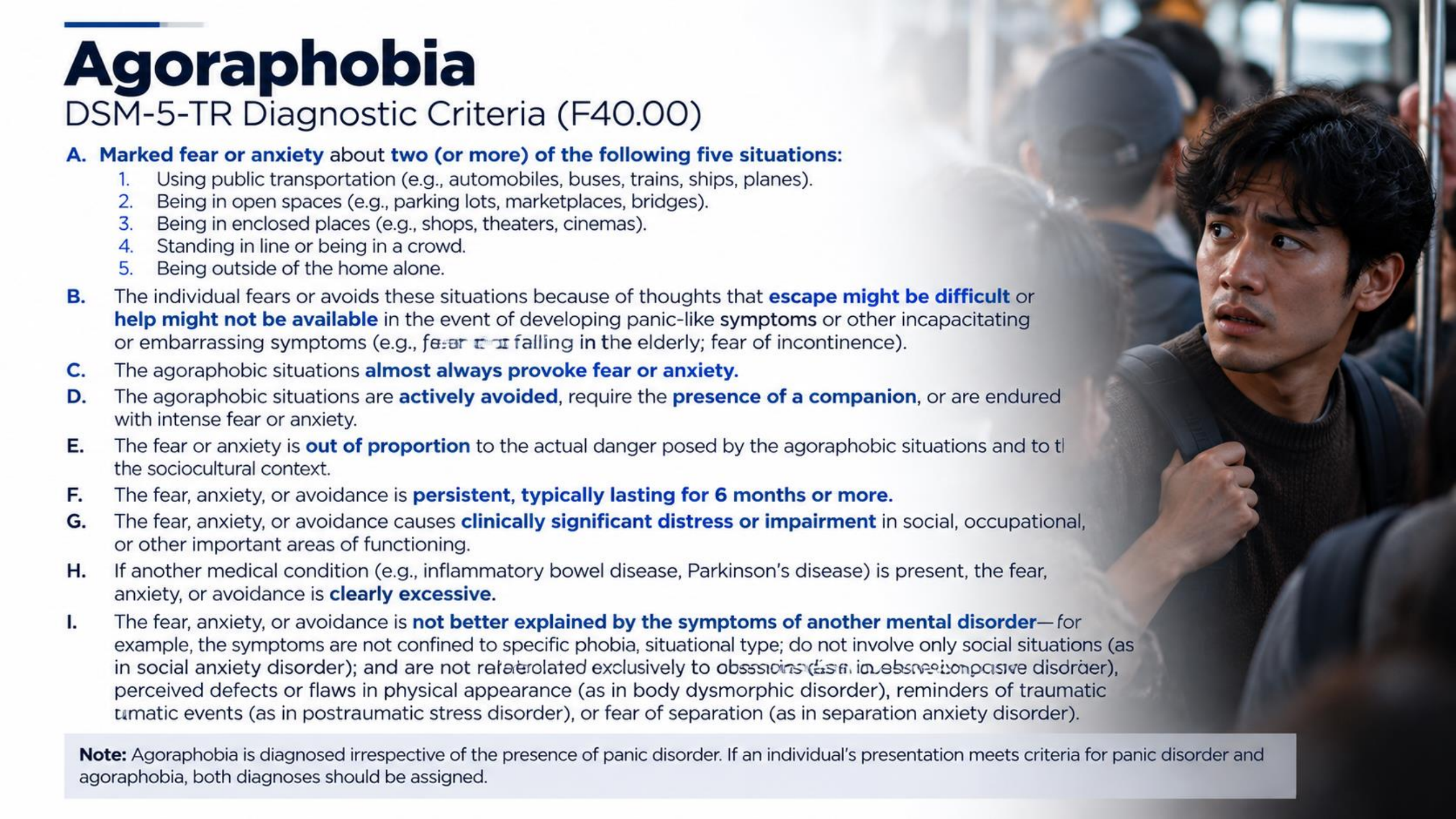
E. The fear or anxiety is **out of proportion** to the actual danger posed by the agoraphobic situations and to t the sociocultural context.

F. The fear, anxiety, or avoidance is **persistent, typically lasting for 6 months or more.**

G. The fear, anxiety, or avoidance causes **clinically significant distress or impairment** in social, occupational, or other important areas of functioning.

H. If another medical condition (e.g., inflammatory bowel disease, Parkinson's disease) is present, the fear, anxiety, or avoidance is **clearly excessive.**

I. The fear, anxiety, or avoidance is **not better explained by the symptoms of another mental disorder**—*for* example, the symptoms are not confined to specific phobia, situational type; do not involve only social situations (as in social anxiety disorder); and are not related exclusively to obsessions (as in obsessive-compulsive disorder), perceived defects or flaws in physical appearance (as in body dysmorphic disorder), reminders of traumatic t matic events (as in posttraumatic stress disorder), or fear of separation (as in separation anxiety disorder).



Note: Agoraphobia is diagnosed irrespective of the presence of panic disorder. If an individual's presentation meets criteria for panic disorder and agoraphobia, both diagnoses should be assigned.

AGORAPHOBIA

STATISTICS, CAUSES/RISK FACTORS, AND TREATMENT

Statistics

- Approximately **1–2%** of adults experience agoraphobia in a given year.
- More common among **females**.
- Often begins during **late adolescence or early adulthood**.

Causes / Risk Factors

- Genetic and biological vulnerability
- Previous panic attacks or panic disorder
- Fear of bodily sensations
- Traumatic or stressful experiences
- Learning and conditioning
- Cognitive tendency to overestimate danger

Treatment

- **CBT**, especially **exposure therapy**, is a major evidence-based treatment.
- **In vivo exposure**: gradually confronting feared situations.
- **SSRIs/SNRIs** may be used to reduce anxiety.
- Medication and CBT may be **combined** when appropriate.



SPECIFIC PHOBIA

Persistent, excessive fear of a specific object or situation

Examples:

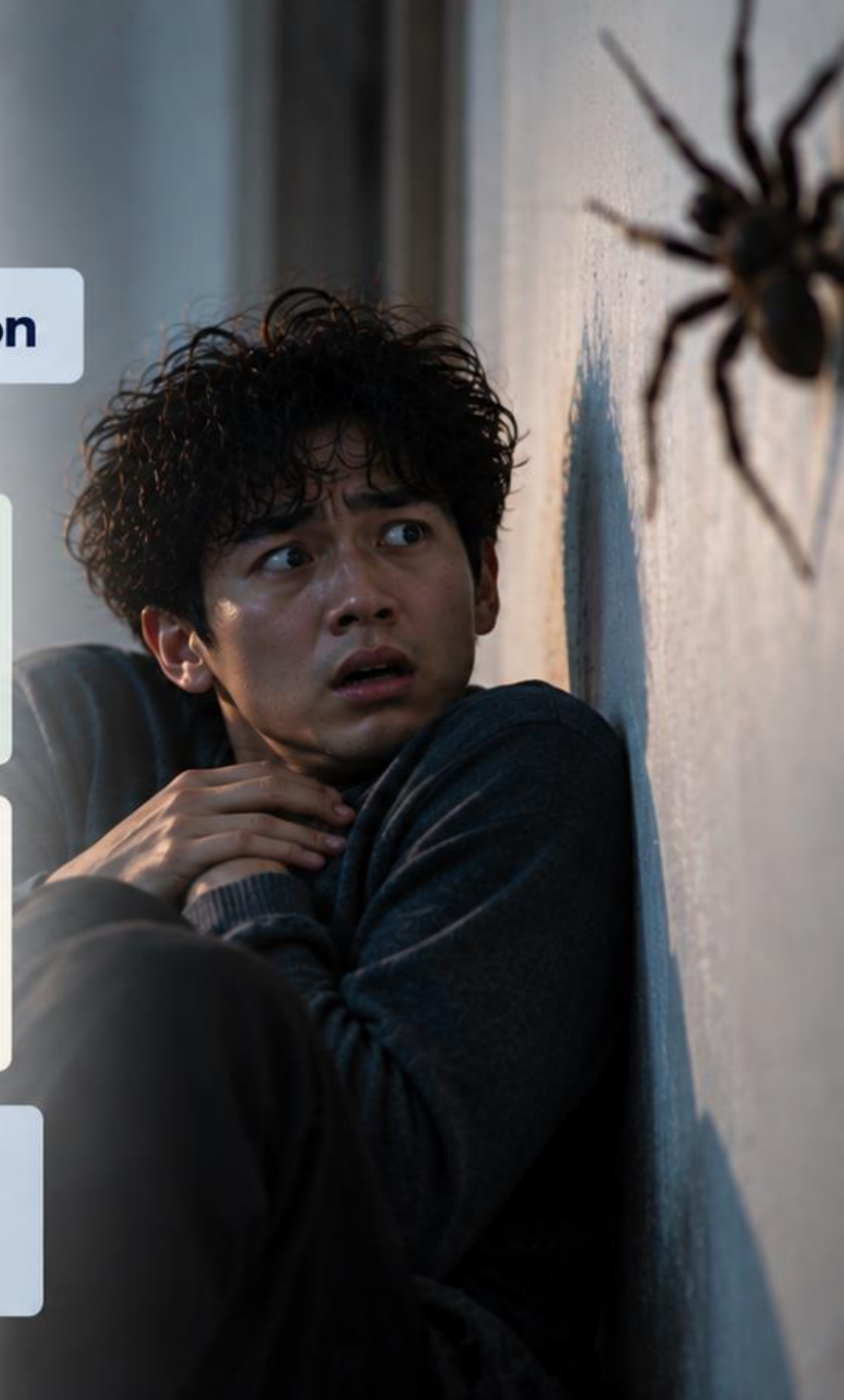
Animals
- spiders
- dogs
- snakes

Natural environment
- heights
- storms
- water

Blood-Injection-Injury
- needles
- blood
- medical procedures

Situational
- flying
- elevators
- driving

COMMON RESPONSE
Fear → Avoidance → Relief → Maintained Fear



THE POWER OF EXPOSURE

Avoidance says:
“Stay away. You’ll be safe.”

Exposure says:
“Approach the feared stimulus in a planned, gradual way.”

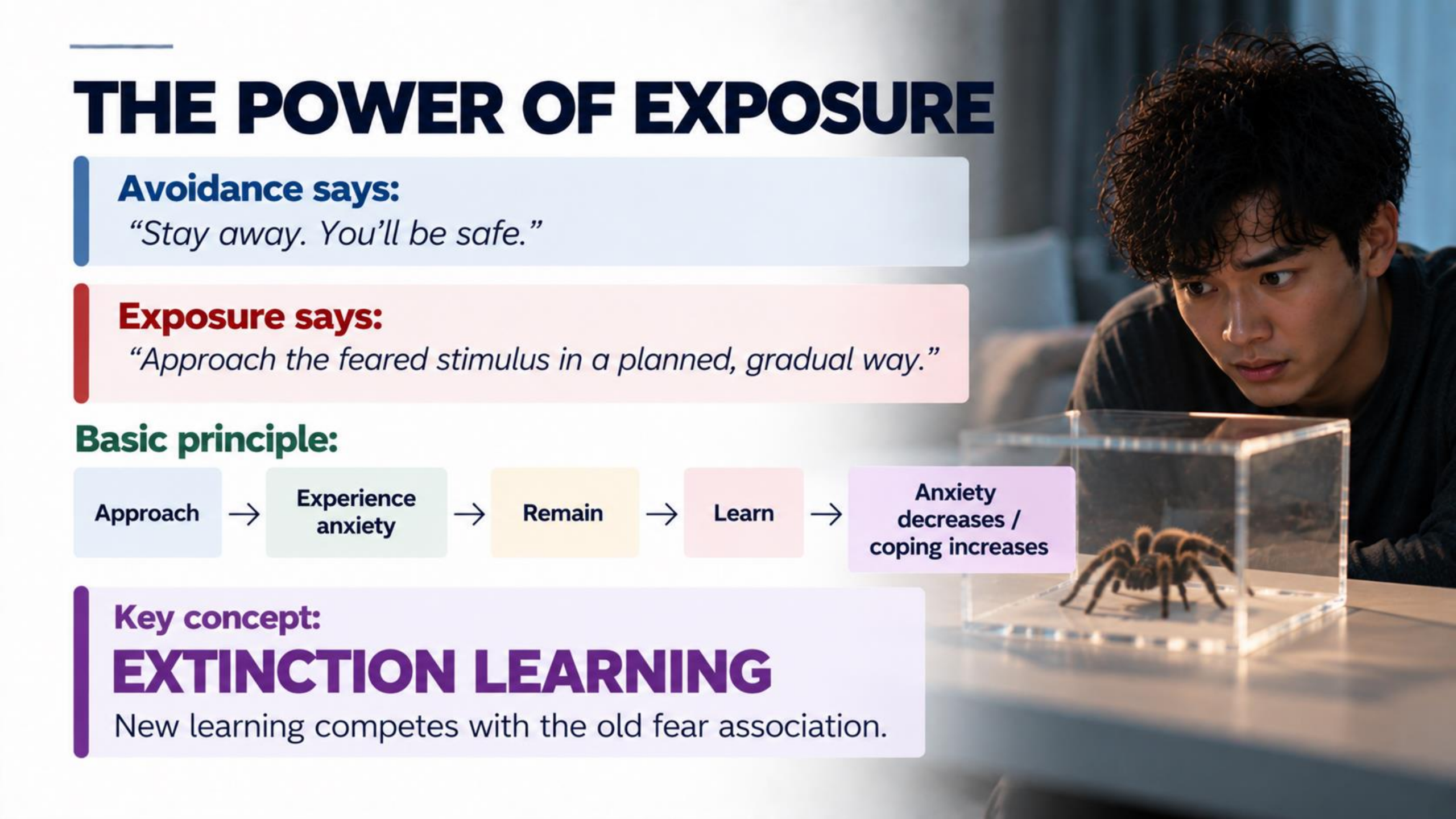
Basic principle:

Approach → Experience anxiety → Remain → Learn → Anxiety decreases / coping increases

Key concept:

EXTINCTION LEARNING

New learning competes with the old fear association.



Specific Phobia

Diagnostic Criteria & Specifiers

Diagnostic criteria

A. Marked fear or anxiety about a specific object or situation (e.g., flying, heights, animals, receiving an injection, seeing blood). In children, the fear or anxiety may be expressed by crying, tantrums, freezing, or clinging.

B. The phobic object or situation almost always provokes immediate fear or anxiety.

C. The phobic object or situation is actively avoided or endured with intense fear or anxiety.

D. The fear or anxiety is out of proportion to the actual danger posed by the specific object or situation and to the sociocultural context.

E. The fear, anxiety, or avoidance is persistent, typically lasting for 6 months or more.

F. The fear, anxiety, or avoidance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

G. The disturbance is not better explained by the symptoms of another mental disorder, including agoraphobia; obsessive-compulsive disorder; posttraumatic stress disorder; separation anxiety disorder; or social anxiety disorder.

Specify by phobic stimulus

F40.218 Animal
e.g., spiders, insects, dogs.

F40.228 Natural environment
e.g., heights, storms, water.

Blood-injection-injury

- **F40.230** Fear of blood.
- **F40.231** Fear of injections and transfusions.
- **F40.232** Fear of other medical care.
- **F40.233** Fear of injury.

F40.248 Situational
e.g., airplanes, elevators, enclosed places.

F40.298 Other
e.g., situations that may lead to choking or vomiting; in children, e.g., loud sounds or costumed characters.

Coding note: When multiple phobic stimuli are present, code each applicable subtype.

SPECIFIC PHOBIA

STATISTICS, CAUSES/RISK FACTORS, AND TREATMENT



Statistics

- Approximately **7–12%** lifetime prevalence.
- More common in **females.**
- Often begins during **childhood.**

Causes / Risk Factors

- Genetic/temperamental vulnerability
- **Direct traumatic experience**
- Observational learning
- Informational learning (hearing about danger)
- Fear conditioning
- Heightened sensitivity to threat

Treatment

- **Exposure therapy** — primary evidence-based treatment.
- **Gradual exposure** or **in vivo exposure**.
- Cognitive-behavioral techniques.
- For blood-injection-injury phobia: **applied tension** may help prevent fainting.

SOCIAL ANXIETY DISORDER

Core fear:

"Negative evaluation by other people"

Common feared situations:

- speaking in class
- meeting unfamiliar people
- eating in public
- performing
- being observed
- interacting with authority figures

Typical thoughts:

"They'll think I'm incompetent."
"I'll embarrass myself."
"Everyone will notice my anxiety."



SAFETY BEHAVIORS

People may attempt to prevent embarrassment by:

- avoiding eye contact
- rehearsing excessively
- speaking very quietly
- avoiding participation
- staying close to familiar people
- mentally reviewing everything they said

THE PROBLEM

Safety behavior can prevent the person from learning:

"I can cope even if I feel anxious."



Social Anxiety Disorder

DSM-5-TR Diagnostic Criteria (F40.10)

A. **Marked fear or anxiety** about one or more social situations in which the individual is exposed to possible **scrutiny** by others. Examples include social interactions (e.g., having a conversation, meeting unfamiliar people), being observed (e.g., eating or drinking), and performing in front of others (e.g., giving a speech).
Note: In children, the anxiety must occur in peer settings and not just during interactions with adults.

B. The individual fears that he or she will act in a way or show anxiety symptoms that will be **negatively evaluated** (i.e., will be humiliating or embarrassing; will lead to rejection or offend others).

C. The social situations **almost always provoke fear or anxiety**.
Note: In children, the fear or anxiety may be expressed by crying, tantrums, freezing, clinging, shrinking, or failing to speak in social situations.

D. The social situations are **avoided or endured** with intense fear or anxiety.

E. The fear or anxiety is **out of proportion** to the actual threat posed by the social situation and to the sociocultural context.

F. The fear, anxiety, or avoidance is **persistent, typically lasting for 6 months or more**.

G. The fear, anxiety, or avoidance causes **clinically significant distress or impairment** in social, occupational, or other important areas of functioning.

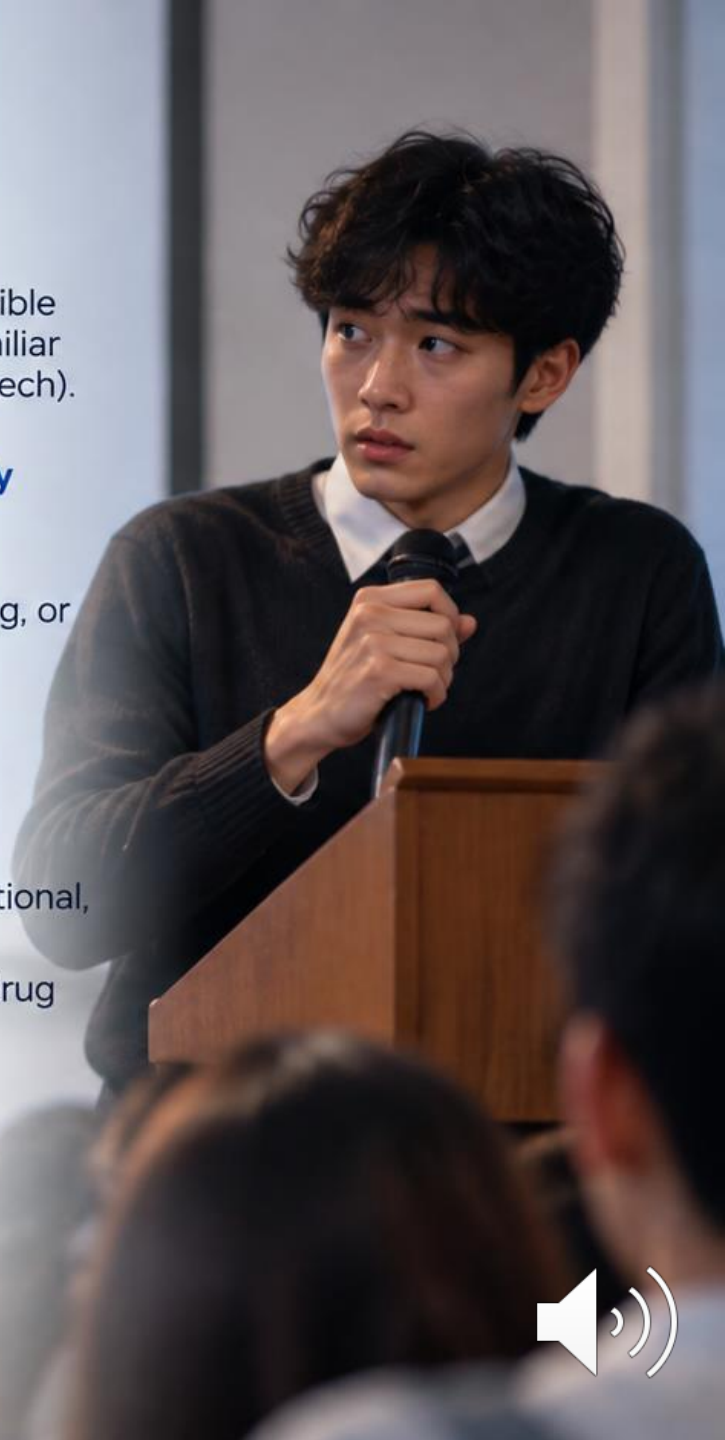
H. The fear, anxiety, or avoidance is **not attributable** to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

I. The fear, anxiety, or avoidance is **not better explained** by the symptoms of another mental disorder, such as panic disorder, body dysmorphic disorder, or autism spectrum disorder.

J. If another medical condition (e.g., Parkinson's disease, obesity, disfigurement from burns or injury) is present, the fear, anxiety, or avoidance is **clearly unrelated or is excessive**.

Specify if:

- **Performance only**: If the fear is restricted to speaking or performing in public.



SOCIAL ANXIETY DISORDER

STATISTICS, CAUSES/RISK FACTORS, AND TREATMENT

Statistics

- Approximately **7%** lifetime prevalence in U.S. adults.
- More common among **females.**
- Often begins during **childhood or adolescence.**

Causes / Risk Factors

- Genetic vulnerability
- Temperamental factors such as **behavioral inhibition.**
- Negative social experiences, bullying, or rejection
- Family/environmental factors
- **Fear of negative evaluation.**
- Cognitive biases toward interpreting social situations negatively

Treatment

- **CBT with exposure** — first-line psychological treatment.
- **SSRIs/SNRIs** may be used.
- **Social-skills training** may be incorporated.
- **CBT + medication** may be appropriate for some individuals.



WHAT IS THE PERSON AFRAID OF?

Disorder	Central Concern
GAD	Multiple everyday threats / uncertainty
Panic Disorder	Panic attacks and their consequences
Agoraphobia	Difficulty escaping or obtaining help
Specific Phobia	Specific object or situation
Social Anxiety Disorder	Negative evaluation



Other Anxiety Disorders

1. **Separation Anxiety Disorder** – Excessive fear or anxiety about separation from attachment figures, leading to significant distress or impairment.
2. **Selective Mutism** – Consistent inability to speak in specific social situations despite being able to speak in other situations.
3. **Panic Attack Specifier** – A sudden surge of intense fear or discomfort that reaches a peak within minutes and involves physical and cognitive symptoms; it can occur across different disorders.
4. **Substance/Medication-Induced Anxiety Disorder** – Significant anxiety or panic symptoms that develop as a direct physiological effect of a substance, medication, intoxication, or withdrawal.
5. **Anxiety Disorder Due to Another Medical Condition** – Significant anxiety or panic symptoms that are directly caused by the physiological effects of another medical condition.
6. **Other Specified Anxiety Disorder** – Clinically significant anxiety symptoms that cause distress or impairment but do not fully meet the criteria for a specific anxiety disorder, with the clinician specifying why.
7. **Unspecified Anxiety Disorder** – Clinically significant anxiety symptoms that cause distress or impairment but do not fully meet the criteria for a specific anxiety disorder, without specifying the reason, typically because there is insufficient information.



Separation Anxiety Disorder

DSM-5-TR Diagnostic Criteria (F93.0)

A. Developmentally inappropriate and excessive fear or anxiety concerning separation from those to whom the individual is attached, as evidenced by **at least three** of the following:

1. Recurrent excessive distress when anticipating or experiencing separation from home or from major attachment figures.
2. Persistent and excessive worry about losing major attachment figures or about possible harm to them, such as illness, injury, disasters, or death.
3. Persistent and excessive worry about experiencing an untoward event (e.g., getting lost, being kidnapped, having an accident, becoming ill) that causes separation from a major attachment figure.
4. Persistent reluctance or refusal to go out, away from home, to school, to work, or elsewhere because of fear of separation.
5. Persistent and excessive fear of or reluctance about being alone or without major attachment figures at home or in other settings.
6. Persistent reluctance or refusal to sleep away from home or to go to sleep without being near a major attachment figure.
7. Repeated nightmares involving the theme of separation.
8. Repeated complaints of physical symptoms (e.g., headaches, stomachaches, nausea, vomiting) when separation from major attachment figures occurs or is anticipated.

B. The fear, anxiety, or avoidance is persistent, lasting **at least 4 weeks in children and adolescents and typically 6 months or more in adults.**

C. The disturbance causes **clinically significant distress or impairment** in social, academic, occupational, or other important areas of functioning.

D. The disturbance is **not better explained by another mental disorder**, such as refusing to leave home because of excessive resistance to change in autism spectrum disorder; delusions or hallucinations concerning separation in psychotic disorders; refusal to go outside without a trusted companion in agoraphobia; worries about ill health or other harm befalling significant others in generalized anxiety disorder; or concerns about having an illness in illness anxiety disorder.



Selective Mutism

DSM-5-TR Diagnostic Criteria (F94.0)

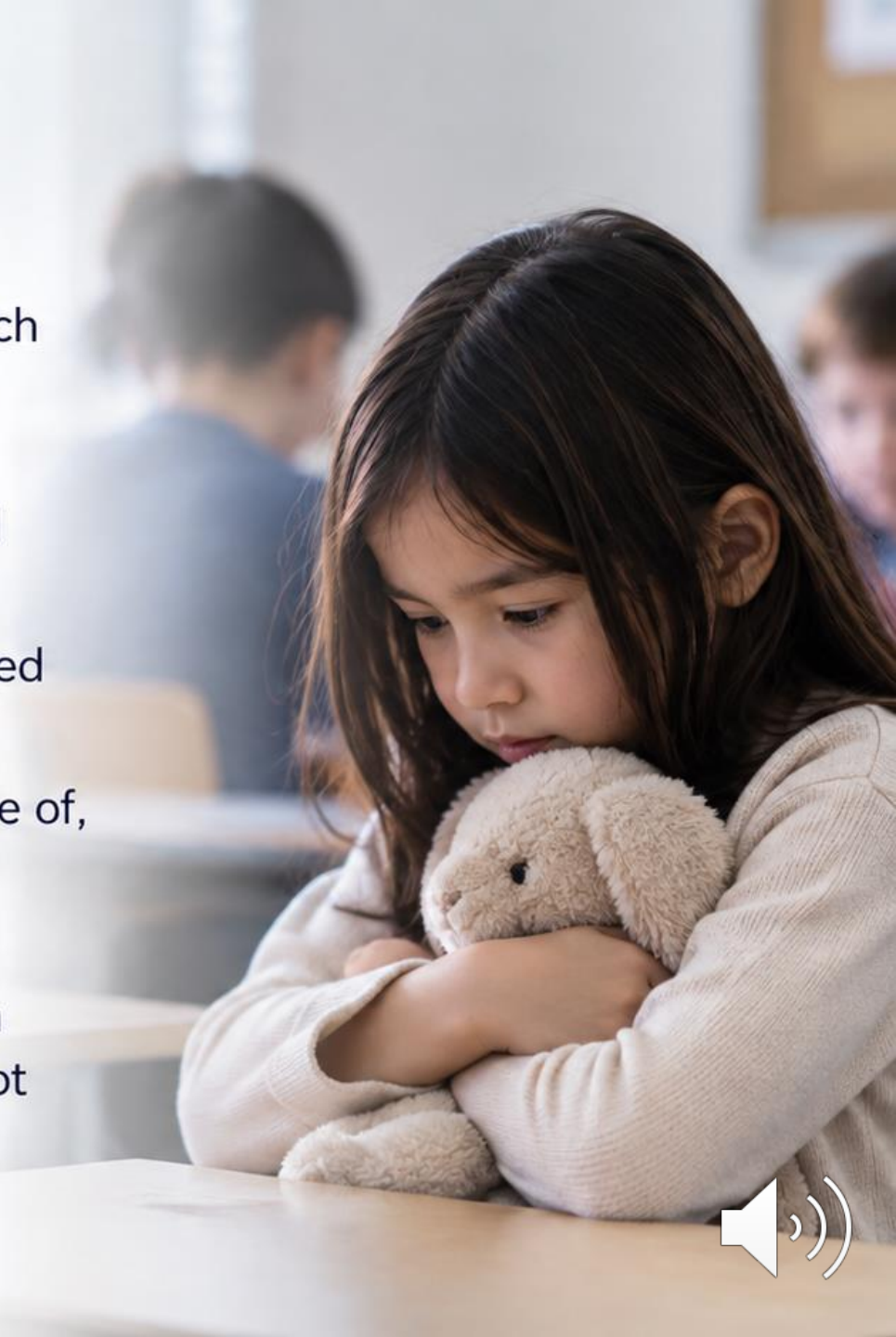
A. Consistent failure to speak in specific social situations in which there is an expectation for speaking (e.g., at school) despite speaking in other situations.

B. The disturbance interferes with educational or occupational achievement or with social communication.

C. The duration of the disturbance is at least 1 month (not limited to the first month of school).

D. The failure to speak is not attributable to a lack of knowledge of, or comfort with, the spoken language required in the social situation.

E. The disturbance is not better explained by a communication disorder (e.g., childhood-onset fluency disorder) and does not occur exclusively during the course of autism spectrum disorder, schizophrenia, or another psychotic disorder.



Panic Attack Specifier

Note: Symptoms are presented for the purpose of identifying a panic attack; however, panic attack is not a mental disorder and cannot be coded. Panic attacks can occur in the context of any anxiety disorder as well as other mental disorders (e.g., depressive disorders, posttraumatic stress disorder, substance use disorders) and some medical conditions (e.g., cardiac, respiratory, vestibular, gastrointetinal). When the presence of a panic attack is identified, it should be noted as a specifier (e.g., "posttraumatic stress disorder with panic attacks"). For panic disorder, the presence of a panic attack is contained within the criteria for the disorder and panic attack is not used as a specifier.

An abrupt surge of intense fear or intense discomfort that reaches a peak within minutes, and during which time **four (or more)** of the following symptoms occur:

Note: The abrupt surge can occur from a calm state or an anxious state.

1. Palpitations, pounding heart, or accelerated heart rate.
2. Sweating.
3. Trembling or shaking.
4. Sensations of shortness of breath or smothering.
5. Feelings of choking.
6. Chest pain or discomfort.
7. Nausea or abdominal distress.
8. Feeling dizzy, unsteady, light-headed, or faint.
9. Chills or heat sensations.
10. Paresthesias (numbness or tingling sensations).
11. Derealization (feelings of unreality) or depersonalization (being detached from oneself).
12. Fear of losing control or "going crazy."
13. Fear of dying.

Note: Culture-specific symptoms (e.g., tinnitus, neck soreness, headache, uncontrollable screaming or crying) may be seen. Such symptoms should not count as one of the four required symptoms.



Substance/Medication-Induced Anxiety Disorder

Diagnostic Criteria

Diagnostic criteria

A. Panic attacks or anxiety is predominant in the clinical picture.

B. There is evidence from the history, physical examination, or laboratory findings of both (1) and (2):
   1. The symptoms in Criterion A developed during or soon after substance intoxication or withdrawal or after exposure to or withdrawal from a medication.
   2. The involved substance/medication is capable of producing the symptoms in Criterion A.

C. The disturbance is **not** better explained by an anxiety disorder that is not substance/medication-induced. Such evidence of an independent anxiety disorder could include:
   - Symptoms precede the onset of substance/medication use.
   - Symptoms persist for a substantial period of time (e.g., about 1 1 month) after cessation of acute withdrawal or severe intoxication.
   - Other evidence suggesting an independent non-substance/medication-induced anxiety disorder.

D. The disturbance does not occur exclusively during the course of a delirium.

E. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Note: This diagnosis should be made instead of a diagnosis of substance intoxication or substance withdrawal only when the symptoms in Criterion A predominate in the clinical picture and they are sufficiently severe to warrant clinical attention.

ICD-10-CM Coding

Substance/Medication	With mild use disorder	With moderate or severe use disorder	Without use disorder
Alcohol	F10.180	F10.280	F10.980
Caffeine	NA	NA	F15.980
Cannabis	F12.180	F12.280	F12.980
Phencyclidine	F16.180	F16.280	F16.980
Other hallucinogen	F16.180	F16.280	F16.980
Inhalant	F18.180	F18.280	F18.980
Opioid	F11.188	F11.288	F11.988
Sedative, hypnotic, or anxiolytic	F13.180	F13.280	F13.980
Amphetamine-type substance (or other stimulant)	F15.180	F15.280	F15.980
Cocaine	F14.180	F14.280	F14.980
Other (or unknown) substance	F19.180	F19.280	F19.980

Specify (onset)

With onset during intoxication: If criteria are met for intoxication with the substance and the symptoms develop during intoxication.

With onset during withdrawal: If criteria are met for withdrawal from the substance and the symptoms develop during, or shortly after, withdrawal.

With onset after medication use: If symptoms developed at initiation of medication, with a change in use of medication, or during withdrawal of medication.

Anxiety Disorder Due to Another Medical Condition

DSM-5-TR Diagnostic Criteria (F06.4)

A. **Panic attacks or anxiety** is predominant in the clinical picture.

B. There is evidence from the history, physical examination, or laboratory findings that the disturbance is the **direct pathophysiological consequence of another medical condition**.

C. The disturbance is **not better explained by another mental disorder**.

D. The disorder **does not occur exclusively during the course of a delirium**.

E. The disturbance causes **clinically significant distress or impairment** in social, occupational, or other important areas of functioning.

Coding note: Include the name of the other medical condition within the name of the mental disorder (e.g., F06.4 anxiety disorder due to pheochromocytoma). The other medical condition should be coded and listed separately immediately before the anxiety disorder due to the medical condition (e.g., D35.00 pheochromocytoma; F06.4 anxiety disorder due to pheochromocytoma).



Other Specified Anxiety Disorder

DSM-5-TR (F41.8)

This category applies to presentations in which symptoms characteristic of an anxiety disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the anxiety disorders diagnostic class, and do not meet criteria for adjustment disorder with anxiety or adjustment disorder with mixed anxiety and depressed mood.

The other specified anxiety disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific anxiety disorder. This is done by recording "other specified anxiety disorder" followed by the specific reason (e.g., "generalized anxiety occurring less often than 'more days than not'").

Examples of presentations that can be specified using the "other specified" designation include the following:

1. **Limited-symptom attacks.**
2. **Generalized anxiety occurring less often than "more days than not."**
3. ***Khyâl cap* (wind attacks):** See "Culture and Psychiatric Diagnosis" in Section III.
4. ***Ataque de nervios* (attack of nerves):** See "Culture and Psychiatric Diagnosis" in Section III.



Unspecified Anxiety Disorder

DSM-5-TR (F41.9)

This category applies to presentations in which symptoms characteristic of an anxiety disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning **predominate but do not meet the full criteria** for any of the disorders in the anxiety disorders diagnostic class, and do not meet criteria for adjustment disorder with anxiety or adjustment disorder with mixed anxiety and depressed mood.

The unspecified anxiety disorder category is used in situations in which the clinician chooses **not to specify** the reason that the criteria are not met for a specific anxiety disorder and includes presentations in which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).



TRAUMA- AND STRESSOR-RELATED DISORDERS



TRAUMA- AND STRESSOR-RELATED DISORDERS

WHEN THE THREAT HAS ALREADY HAPPENED

Trauma-related disorders differ from many anxiety disorders because the symptoms are organized around exposure to a **traumatic or stressful event.**

Focus of today's discussion:

POSTTRAUMATIC STRESS DISORDER

Trauma can affect:

Memory • Emotion • Cognition • Behavior • Arousal • Relationships



POSTTRAUMATIC STRESS DISORDER

Following exposure to a traumatic event, symptoms may involve:

1. INTRUSION

- unwanted memories
- nightmares
- flashbacks
- distress to reminders

2. AVOIDANCE

- avoiding memories
- avoiding places, people, activities, or reminders

3. NEGATIVE CHANGES

- negative beliefs
- persistent negative emotional states
- detachment
- altered interpretations

4. AROUSAL / REACTIVITY

- hypervigilance
- exaggerated startle
- irritability
- sleep problems
- concentration difficulties



PTSD: WHY DOES IT PERSIST?

THE TRAUMA RESPONSE

TRAUMA

↓

Threat system becomes highly sensitized

↓

Intrusive memories / reminders

↓

Avoidance

↓

Short-term reduction in distress

↓

Limited opportunity for corrective learning

↓

Symptoms may persist

Other factors:

- biological vulnerability
- prior experiences
- cognitive appraisals
- social support
- coping resources

POSTTRAUMATIC STRESS DISORDER

Diagnostic Criteria • Adults, Adolescents, and Children Older Than 6 Years

A. TRAUMATIC EXPOSURE

Exposure to actual or threatened death, serious injury, or sexual violence in one (or more) of the following ways:

1. Directly experiencing the traumatic event(s).
2. Witnessing, in person, the event(s) as it occurred to others.
3. Learning that the event(s) occurred to a close family member or close friend (if death, the event must have been violent or accidental).
4. Experiencing repeated or extreme exposure to aversive details of the event(s) (e.g., first responders, police officers).

Note: Exposure through electronic media, television, movies, or pictures does not apply unless this exposure is work related.

B. INTRUSION SYMPTOMS

One (or more) of the following, beginning after the traumatic event(s) occurred:

1. Recurrent, involuntary, and intrusive distressing memories.
2. Recurrent distressing dreams related to the event(s).
3. Dissociative reactions (e.g., flashbacks) in which the individual feels or acts as if the event(s) are recurring.
4. Intense or prolonged psychological distress at exposure to reminders.
5. Marked physiological reactions to reminders.

Note: In children older than 6 years, repetitive play may occur, and in younger children, frightening dreams without recognizable content may be present.

C. AVOIDANCE

Persistent avoidance of stimuli associated with the traumatic event(s), beginning after the event(s) occurred, as evidenced by one or both of the following:

1. Avoidance of or efforts to avoid distressing memories, thoughts, or feelings about or closely associated with the event(s).
2. Avoidance of or efforts to avoid external reminders (people, places, conversations, activities, objects, situations) that arouse distressing memories, thoughts, or feelings about or closely associated with the event(s).

D. NEGATIVE COGNITIONS & MOOD

Two (or more) of the following, beginning or worsening after the event(s) occurred:

1. Inability to remember an important aspect of the event(s).
2. Persistent and exaggerated negative beliefs or expectations about oneself, others, or the world.
3. Persistent, distorted cognitions about the cause or consequences, leading to blame.
4. Persistent negative emotional state (e.g., fear, horror, anger, guilt, or shame).
5. Markedly diminished interest or participation in significant activities.
6. Feelings of detachment or estrangement from others.
7. Persistent inability to experience positive emotions (e.g., happiness, satisfaction, or love).

E. AROUSAL & REACTIVITY

Two (or more) of the following, beginning or worsening after the event(s) occurred:

1. Irritable behavior and angry outbursts (with little or no provocation).
2. Reckless or self-destructive behavior.
3. Hypervigilance.
4. Exaggerated startle response.
5. Problems with concentration.
6. Sleep disturbance (e.g., difficulty falling or staying asleep or restless sleep).



Full diagnosis also requires duration > 1 month (F), clinically significant distress or impairment (G), and symptoms not attributable to substances or another medical condition (H).

POSTTRAUMATIC STRESS DISORDER

Diagnostic Criteria • Duration, Impairment & Specifiers

F. DURATION

The disturbance (Criteria B, C, D, and E) lasts more than 1 month.

G. CLINICAL SIGNIFICANCE

The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

H. EXCLUSION

The disturbance is not attributable to the physiological effects of a substance (e.g., medication, alcohol) or another medical condition.

SPECIFY WHETHER

WITH DISSOCIATIVE SYMPTOMS

The individual meets PTSD criteria and experiences persistent or recurrent:

1. **Depersonalization** — feeling detached from, or like an outside observer of, one's mental processes or body; experiences may include feeling in a dream or that the self/body is unreal.
2. **Derealization** — persistent or recurrent experiences that surroundings are unreal, dreamlike, distant, or distorted.

> **Note:** These symptoms must not be attributable to substances or another medical condition.

WITH DELAYED EXPRESSION

Full diagnostic criteria are not met until at least 6 months after the event, although some symptoms may begin immediately.



For clinical use, apply the complete diagnostic criteria and consider developmental presentation.

PTSD in Children 6 Years and Younger

A. Trauma Exposure

Exposure to actual or threatened death, serious injury, or sexual violence through:

1. Direct experience of the traumatic event(s).
2. Witnessing, in person, the event(s), especially involving a primary caregiver.
3. Learning that the event(s) happened to a parent or caregiver.

B. Intrusion Symptoms

At least one symptom beginning after the trauma:

1. Recurrent, involuntary, and intrusive distressing memories (may appear as play reenactment).
2. Recurrent distressing dreams related to the event(s) (content may be unclear).
3. Dissociative reactions (e.g., flashbacks); trauma-specific reenactment may occur in play.
4. Intense or prolonged psychological distress at trauma reminders.
5. Marked physiological reactions to reminders.

C. Avoidance and Negative Changes

At least one symptom, beginning or worsening after the event:

Persistent Avoidance

1. Avoidance of or efforts to avoid activities, places, or physical reminders that arouse recollections of the traumatic event(s).
2. Avoidance of or efforts to avoid people, conversations, or interpersonal situations that arouse recollections of the traumatic event(s).

Negative Alterations in Cognitions and Mood

3. Substantially increased frequency of negative emotional states (e.g., fear, guilt, sadness, shame, confusion).
4. Markedly diminished interest or participation in significant activities, including constriction of play.
5. Socially withdrawn behavior.
6. Persistent reduction in expression of positive emotions.

PTSD in Children 6 Years and Younger

D. Arousal and Reactivity

Two (or more) symptoms, beginning or worsening after the traumatic event(s):

1. Irritable behavior and angry outbursts (with little or no provocation), typically expressed as verbal or physical aggression toward people or objects (including extreme temper tantrums).
2. Hypervigilance.
3. Exaggerated startle response.
4. Problems with concentration.
5. Sleep disturbance (e.g., difficulty falling or staying asleep or restless sleep).

E. Duration

The duration of the disturbance is more than 1 month.

F. Impact

The disturbance causes clinically significant distress or impairment in relationships with parents, siblings, peers, or other caregivers or with school behavior.

G. Exclusion

The disturbance is not attributable to the physiological effects of a substance (e.g., medication or alcohol) or another medical condition.

Specify Whether

With dissociative symptoms

The individual's symptoms meet the criteria for PTSD, and the individual experiences persistent or recurrent symptoms of either of the following:

1. **Depersonalization:** Persistent or recurrent experiences of feeling detached from, and as if one were an outside observer of, one's mental processes or body (e.g., feeling as though one were in a dream; feeling a sense of unreality of self or body or of time moving slowly).
2. **Derealization:** Persistent or recurrent experiences of unreality of surroundings (e.g., the world around the individual is experienced as unreal, dreamlike, distant, or distorted).

Note: To use this subtype, the dissociative symptoms must not be attributable to the physiological effects of a substance (e.g., blackouts) or another medical condition (e.g., complex partial seizures).

Specify if: With delayed expression

If the full diagnostic criteria are not met until at least 6 months after the event (although the onset and expression of some symptoms may be immediate).



Posttraumatic Stress Disorder (PTSD)

1. Statistics

- Approximately **6%** of U.S. adults experience PTSD during their lifetime.
- PTSD is **more common in women** than men.
- Not everyone exposed to trauma develops PTSD.

2. Causes/Risk Factors

- Exposure to trauma, such as abuse, accidents, combat, or disasters.
- Previous traumatic experiences or childhood abuse.
- Preexisting mental health problems.
- Limited social support.
- Severe, prolonged, or repeated trauma.

3. Treatment

- **Cognitive Processing Therapy (CPT):** Changes unhelpful trauma-related beliefs.
- **Prolonged Exposure (PE):** Gradually confronts traumatic memories and avoided situations.
- **EMDR:** Helps reduce distress associated with traumatic memories.
- **Medication:** Antidepressants, such as sertraline, may reduce symptoms.
- **Social Support:** Provides emotional support during recovery.

Other Trauma- and Stressor-Related Disorders

1. **Reactive Attachment Disorder (RAD)** – Characterized by emotionally withdrawn behavior toward caregivers and limited seeking or response to comfort, usually following severe neglect or insufficient care.
2. **Disinhibited Social Engagement Disorder (DSED)** – Characterized by overly familiar behavior and reduced caution toward unfamiliar adults, associated with severe insufficient care.
3. **Acute Stress Disorder (ASD)** – Trauma-related symptoms that occur after a traumatic event and last from 3 days to 1 month.
4. **Adjustment Disorders** – Emotional or behavioral symptoms that develop in response to an identifiable stressor and cause significant distress or impairment.
5. **Prolonged Grief Disorder (PGD)** – Persistent, intense grief following the death of a loved one that significantly interferes with daily functioning.
6. **Other Specified Trauma- and Stressor-Related Disorder** – Clinically significant trauma- or stressor-related symptoms that do not meet the full criteria for a specific disorder, with the reason specified.
7. **Unspecified Trauma- and Stressor-Related Disorder** – Clinically significant trauma- or stressor-related symptoms that do not meet the full criteria for a specific disorder, with the reason unspecified.

Reactive Attachment Disorder (RAD)

DSM-5-TR code: 313.89 (F94.1)

Diagnostic Criteria

1 **Emotionally withdrawn, inhibited behavior toward adult caregivers.**

- The child rarely or minimally seeks comfort when distressed.
- The child rarely or minimally responds to comfort when distressed.

2 **Persistent social and emotional disturbance with at least two of the following:**

- Minimal social and emotional responsiveness to others.
- Limited positive affect.
- Episodes of unexplained irritability, sadness, or fearfulness that are evident even during nonthreatening interactions with adult caregivers.

3 **History of extreme insufficient care, as evidenced by at least one of the following:**

- Social neglect or deprivation (persistent lack of basic emotional needs for comfort, stimulation, and affection met by caregiving adults).
- Repeated changes of primary caregivers that limit opportunities to form stable attachments (e.g., frequent changes in foster care).
- Rearing in unusual settings that severely limit opportunities to form selective attachments (e.g., institutions with high child-to-caregiver ratios).

4 The insufficient care in Criterion 3 is presumed to be responsible for the disturbed behavior in Criteria 1 and 2.

5 The criteria are not met for autism spectrum disorder.

6 The disturbance is evident before age 5 years.

7 The child has a developmental age of at least 9 months.

*Specify **persistent*** if the disorder has been present for more than 12 months; *specify **severe*** when all symptoms are present at high levels.

Disinhibited Social Engagement Disorder (DSED)

DSM-5-TR: 313.89 (F94.2)

Diagnostic Criteria

A **A pattern of behavior in which a child actively approaches and interacts with unfamiliar adults and exhibits at least two of the following:**

1. Reduced or absent reticence in approaching and interacting with unfamiliar adults.
2. Overly familiar verbal or physical behavior (that is not consistent with culturally sanctioned and with age-appropriate social boundaries).
3. Diminished or absent checking back with adult caregiver after venturing away, even in unfamiliar settings.
4. Willingness to go off with an unfamiliar adult with minimal or no hesitation.

B **The behaviors in Criterion A are not limited to impulsivity (as in attention-deficit/hyperactivity disorder) but include socially disinhibited behavior.**

C **The child has experienced a pattern of extremes of insufficient care as evidenced by at least one of the following:**

1. Social neglect or deprivation in the form of persistent lack of having basic emotional needs for comfort, stimulation, and affection met by caregiving adults.
2. Repeated changes of primary caregivers that limit opportunities to form stable attachments (e.g., frequent changes in foster care).
3. Rearing in unusual settings that severely limit opportunities to form selective attachments (e.g., institutions with high child-to-caregiver ratios).

D **The care in Criterion C is presumed to be responsible for the disturbed behavior in Criterion A.**

E **The child has a developmental age of at least 9 months.**

Persistent: The disorder has been present for more than 12 months.

Severe: The disorder is specified as severe when the child exhibits all symptoms of the disorder, with each symptom manifesting at relatively high levels.

Acute Stress Disorder

Diagnostic Criteria F43.0

A. Exposure to actual or threatened death, serious injury, or sexual violence in one (or more) of the following ways:

1. Directly experiencing the traumatic event(s).
2. Witnessing, in person, the event(s) as it occurred to others.
3. Learning that the event(s) occurred to a close family member or close friend.
   Note: In cases of actual or threatened death of a family member or friend, the event(s) must have been violent or accidental.
4. Experiencing repeated or extreme exposure to aversive details of the traumatic event(s) (e.g., first responders collecting human remains, police officers repeatedly exposed to details of child abuse).

> **Note:** This does not apply to exposure through electronic media, television, movies, or pictures, unless this exposure is work related.

Acute Stress Disorder

Criterion B • Symptom Clusters

B. Presence of nine (or more) symptoms from any of the five categories of intrusion, negative mood, dissociation, avoidance, and arousal, beginning or worsening after the traumatic event(s) occurred.

Intrusion Symptoms

1. Recurrent, involuntary, intrusive distressing memories; in children, repetitive play may express trauma themes.
2. Recurrent distressing dreams related to the event; children may have frightening dreams without recognizable content.
3. Dissociative reactions/flashbacks, as if the event is recurring; children may reenact trauma in play.
4. Intense psychological distress or marked physiological reactions to internal or external trauma reminders.

Negative Mood

5. Persistent inability to experience positive emotions, such as happiness, satisfaction, or loving feelings.

Note: At least 9 symptoms across the five categories are required.

Dissociative Symptoms

6. Altered sense of reality of oneself or surroundings (e.g., feeling in a daze or time slowing).
7. Inability to remember an important aspect of the trauma, typically due to dissociative amnesia.

Avoidance Symptoms

8. Efforts to avoid distressing memories, thoughts, or feelings related to the trauma.
9. Efforts to avoid external reminders, such as people, places, conversations, activities, objects, or situations.

Arousal Symptoms

10. Sleep disturbance.
11. Irritable behavior or angry outbursts, sometimes involving verbal or physical aggression.
12. Hypervigilance.
13. Problems with concentration.
14. Exaggerated startle response.



Acute Stress Disorder

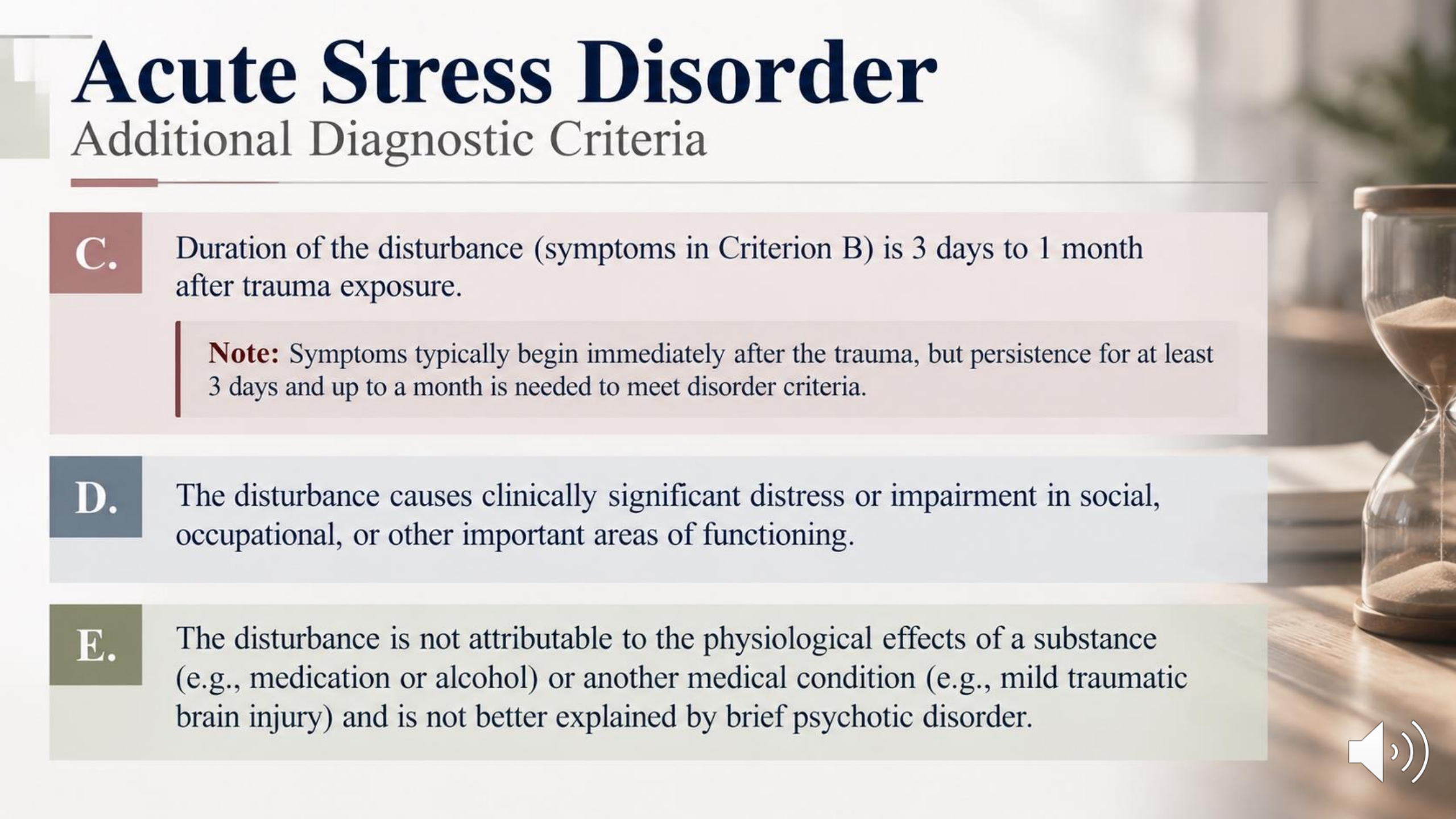
Additional Diagnostic Criteria

C. Duration of the disturbance (symptoms in Criterion B) is 3 days to 1 month after trauma exposure.

> **Note:** Symptoms typically begin immediately after the trauma, but persistence for at least 3 days and up to a month is needed to meet disorder criteria.

D. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., medication or alcohol) or another medical condition (e.g., mild traumatic brain injury) and is not better explained by brief psychotic disorder.



Adjustment Disorders

Diagnostic Criteria

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

1. Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and cultural factors that might influence symptom severity and presentation.
2. Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement and are not better explained by prolonged grief disorder.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional **6 months.**

Specify whether:

F43.21	**With depressed mood:** Low mood, tearfulness, or feelings of hopelessness are predominant.
F43.22	**With anxiety:** Nervousness, worry, jitteriness, or separation anxiety is predominant.
F43.23	**With mixed anxiety and depressed mood:** A combination of depression and anxiety is predominant.
F43.24	**With disturbance of conduct:** Disturbance of conduct is predominant.
F43.25	**With mixed disturbance of emotions and conduct:** Both emotional symptoms (e.g., depression, anxiety) and a disturbance of conduct are predominant.
F43.20	**Unspecified**: For maladaptive reactions that are not classifiable as one of the specific subtypes of adjustment disorder.

Duration Specifiers

Acute	Persistence of symptoms for less than 6 months.
Persistent (chronic)	Persistence of symptoms for 6 months or longer. By definition, symptoms cannot persist for more than 6 months after the termination of the stressor or its consequences. The persistent specifier applies when the disturbance is longer than 6 months in response to a chronic stressor or to a stressor that has enduring consequences.



Prolonged Grief Disorder

Diagnostic Criteria F43.8

A. The death, at least 12 months ago, of a person who was close to the bereaved individual (for children and adolescents, at least 6 months ago).

B. Since the death, the development of a persistent grief response characterized by one or both of the following symptoms, which have been present most days to a clinically significant degree. In addition, the symptom(s) has occurred nearly every day for at least the last month:

1. Intense yearning/longing for the deceased person.
2. Preoccupation with thoughts or memories of the deceased person (in children and adolescents, preoccupation may focus on the circumstances of the death).

C. Since the death, at least three of the following symptoms have been present most days to a clinically significant degree. In addition, the symptoms have occurred nearly every day for at least the last month:

1. Identity disruption (e.g., feeling as though part of oneself has died) since the death.
2. Marked sense of disbelief about the death.
3. Avoidance of reminders that the person is dead (in children and adolescents, may be characterized by efforts to avoid reminders).
4. Intense emotional pain (e.g., anger, bitterness, sorrow) related to the death.
5. Difficulty reintegrating into one's relationships and activities after the death (e.g., problems engaging with friends, pursuing interests, or planning for the future).
6. Emotional numbness (absence or marked reduction of emotional experience) as a result of the death.
7. Feeling that life is meaningless as a result of the death.
8. Intense loneliness as a result of the death.

D. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The duration and severity of the bereavement reaction clearly exceed expected social, cultural, or religious norms for the individual's culture and context.

F. The symptoms are not better explained by another mental disorder, such as major depressive disorder or posttraumatic stress disorder, and are not attributable to the physiological effects of a substance (e.g., medication, alcohol) or another medical condition.

Other Specified Trauma- and Stressor-Related Disorder

This category applies to presentations in which symptoms characteristic of a trauma- and stressor-related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the trauma- and stressor-related disorders diagnostic class. The other specified trauma- and stressor-related disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific trauma- and stressor-related disorder. This is done by recording "other specified trauma- and stressor-related disorder" followed by the specific reason (e.g., "persistent response to trauma with PTSD-like symptoms").

Examples of presentations that can be specified include the following:

1. **Adjustment-like disorders with delayed onset of symptoms** that occur more than 3 months after the stressor.
2. **Adjustment-like disorders with prolonged duration** of more than 6 months without prolonged duration of stressor.
3. **Persistent response to trauma with PTSD-like symptoms** (i.e., symptoms occurring in response to a traumatic event that fall short of the diagnostic threshold for PTSD and that persist for longer than 6 months, sometimes referred to as "subthreshold/partial PTSD").
4. **Ataque de nervios:** See "Culture and Psychiatric Diagnosis" in Section III.
5. **Other cultural syndromes:** See "Culture and Psychiatric Diagnosis" in Section III.



Unspecified Trauma- and Stressor-Related Disorder

F43.9

This category applies to presentations in which symptoms characteristic of a trauma- and stressor-related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the trauma- and stressor-related disorders diagnostic class. The unspecified trauma- and stressor-related disorder category is used in situations in which the clinician chooses not to specify the reason that the criteria are not met for a specific trauma- and stressor-related disorder and includes presentations in which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).

OBSESSIVE-COMPULSIVE DISORDER- AND RELATED DISORDERS



OBSESSIVE-COMPULSIVE DISORDER

OBSESSIONS

Recurrent, intrusive thoughts, urges, or images that are experienced as unwanted and distressing.

COMPULSIONS

Repetitive behaviors or mental acts performed in response to an obsession or according to rigid rules.

THE CYCLE

Obsession

↓

Anxiety / distress

↓

Compulsion

↓

Temporary relief

↺

Cycle reinforced



OCD: Obsession ≠ Compulsion

KNOW THE DIFFERENCE

OBSESSION

The intrusive thought/image/urge

"What if I caused harm?"

≠

COMPULSION

The repetitive response

"I need to check again."

Another example:

Obsession:
"What if I made a mistake?"

Compulsion:
Rechecking the document 20 times.

Important: Not every repetitive behavior is a compulsion.

Obsessive-Compulsive Disorder

Diagnostic Criteria

F42.2

A. Presence of obsessions, compulsions, or both.

Obsessions are defined by (1) and (2):

1. Recurrent and persistent thoughts, urges, or images that are experienced, at some time during the disturbance, as intrusive and unwanted, and that in most individuals cause marked anxiety or distress.
2. The individual attempts to ignore or suppress such thoughts, urges, or images, or to neutralize them with some other thought or action (e.g., by performing a compulsion).

Compulsions are defined by (1) and (2):

1. Repetitive behaviors (e.g., hand washing, ordering, checking) or mental acts (e.g., praying, counting, repeating words silently) that the individual feels driven to perform in response to an obsession or according to rules that must be applied rigidly.
2. The behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation; however, they are not connected in a realistic way with what they are designed to neutralize or prevent, or are clearly excessive.

B. The obsessions or compulsions are time-consuming or cause significant distress or impairment.

The symptoms are time-consuming (e.g., take more than 1 hour per day) or cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

C. Not attributable to a substance or medical condition.

The obsessive-compulsive symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition.

D. Not better explained by another disorder.

The disturbance is not better explained by the symptoms of another mental disorder (e.g., excessive worries in generalized anxiety disorder; appearance concerns in body dysmorphic disorder; difficulty discarding in hoarding disorder; repetitive behaviors in autism spectrum disorder; etc.).

Specify insight level:

With good or fair insight: Recognizes that the OCD beliefs are definitely or probably not true or may or may not be true.
With poor insight: Thinks the OCD beliefs are probably true.
With absent insight/delusional beliefs: Completely convinced that the OCD beliefs are true.

Specify if:

Tic-related: Current or past history of a tic disorder.

Obsessive-Compulsive Disorder (OCD)



Statistics

- Affects approximately 2–3% of people worldwide during their lifetime.
- Often begins in childhood, adolescence, or early adulthood.

Causes/Risk Factors

- Genetic predisposition and family history.
- Differences in brain circuits and neurotransmitter systems.
- Stressful or traumatic experiences.
- Anxiety, intolerance of uncertainty, and environmental factors.

Treatment

- Cognitive Behavioral Therapy (CBT), particularly Exposure and Response Prevention (ERP).
- SSRIs, such as fluoxetine and sertraline, prescribed by a physician.
- Combined psychotherapy and medication for more severe cases.



BODY DYSMORPHIC DISORDER

CENTRAL CONCERN

Preoccupation with perceived flaws or defects in physical appearance that are:

- not observable or
- appear slight to others

Common responses:

- Checking
- Camouflaging
- Comparing
- Reassurance seeking
- Avoidance

Important distinction:
The distress is not simply ordinary dissatisfaction with appearance.

Body Dysmorphic Disorder

Diagnostic Criteria

F45.22

A. Preoccupation with one or more perceived defects or flaws in physical appearance that are not observable or appear slight to others.

B. At some point during the course of the disorder, the individual has performed repetitive behaviors (e.g., mirror checking, excessive grooming, skin picking, reassurance seeking) or mental acts (e.g., comparing his or her appearance with that of others) in response to the appearance concerns.

C. The preoccupation causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

D. The appearance preoccupation is not better explained by concerns with body fat or weight in an individual whose symptoms meet diagnostic criteria for an eating disorder.

Specify if:

With muscle dysmorphia: The individual is preoccupied with the idea that his or her body build is too small or insufficiently muscular. This specifier is used even if the individual is preoccupied with other body areas, which is often the case.

Specify insight level:

With good or fair insight: The individual recognizes that the body dysmorphic disorder beliefs are definitely or probably not true or that they may or may not be true.

With poor insight: The individual thinks that the body dysmorphic disorder beliefs are probably true.

With absent insight/delusional beliefs: The individual is completely convinced that the body dysmorphic disorder beliefs are true.

Body Dysmorphic Disorder (BDD)



Statistics

- Affects approximately 2% of the general population.
- Often begins during adolescence.

Causes/Risk Factors

- Genetic predisposition and family history.
- Negative body image and low self-esteem.
- Bullying, teasing, or appearance-related criticism.
- Perfectionism and sociocultural pressures regarding appearance.

Treatment

- Cognitive Behavioral Therapy (CBT), including exposure and response prevention.
- SSRIs prescribed by a physician.
- Family support and psychoeducation.

OCD-RELATED DISORDERS

HOARDING DISORDER

Persistent difficulty discarding possessions because of perceived need to save them.

TRICHOTILLOMANIA

Recurrent hair pulling resulting in hair loss.

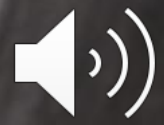
EXCORIATION DISORDER

Recurrent skin picking resulting in tissue damage.



COMMON THEME

Repetitive behavior + difficulty resisting the behavior + significant distress or impairment



THE BEHAVIORAL LOOP

Tension / urge

↓

Repetitive behavior

↓

Temporary relief / gratification

↓

Behavior becomes more likely

↓

Negative consequences

↓

Attempt to resist

↓

URGE RETURNS



Hoarding Disorder

Diagnostic Criteria | F42.3

A. Persistent difficulty discarding or parting with possessions, regardless of their actual value.

B. This difficulty is due to a perceived need to save the items and to distress associated with discarding them.

C. The difficulty discarding possessions results in the accumulation of possessions that congest and clutter active living areas and substantially compromises their intended use. If living areas are uncluttered, it is only because of the interventions of third parties (e.g., family members, cleaners, authorities).

D. The hoarding causes clinically significant distress or impairment in social, occupational, or other important areas of functioning (including maintaining a safe environment for self and others).

E. The hoarding is not attributable to another medical condition (e.g., brain injury, cerebrovascular disease, Prader-Willi syndrome).

F. The hoarding is not better explained by the symptoms of another mental disorder (e.g., obsessions in obsessive-compulsive disorder, decreased energy in major depressive disorder, delusions in schizophrenia or another psychotic disorder, cognitive deficits in major neurocognitive disorder, restricted interests in autism spectrum disorder).

Specify if:

With excessive acquisition: If difficulty discarding possessions is accompanied by excessive acquisition of items that are not needed or for which there is no available space.

Specify insight level:

With good or fair insight: The individual recognizes that hoarding-related beliefs and behaviors (pertaining to difficulty discarding items, clutter, or excessive acquisition) are problematic.

With poor insight: The individual is mostly convinced that these beliefs and behaviors are not problematic despite evidence to the contrary.

With absent insight/delusional beliefs: The individual is completely convinced that these beliefs and behaviors are not problematic despite evidence to the contrary.

Hoarding Disorder



Statistics

- Affects approximately 2–6% of the population.
- Often begins during adolescence and worsens with age.

Causes/Risk Factors

- Genetic predisposition and family history.
- Difficulty making decisions and organizing possessions.
- Emotional attachment to belongings and fear of losing important items.
- Stressful or traumatic life events.

Treatment

- Cognitive Behavioral Therapy (CBT) focused on discarding and organizing.
- Skills training in decision-making and reducing clutter.
- Medication may help manage co-occurring anxiety or depression.

Trichotillomania (Hair-Pulling Disorder)

Diagnostic Criteria | F63.3

A. Recurrent pulling out of one's hair, resulting in hair loss.

B. Repeated attempts to decrease or stop hair pulling.

C. The hair pulling causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

D. The hair pulling or hair loss is not attributable to another medical condition (e.g., a dermatological condition).

E. The hair pulling is not better explained by the symptoms of another mental disorder (e.g., attempts to improve a perceived defect or flaw in appearance in body dysmorphic disorder).

Trichotillomania (Hair-Pulling Disorder)



Statistics

- Affects approximately 1–2% of the population.
- Often begins during adolescence, particularly around puberty.

Causes/Risk Factors

- Genetic predisposition and family history.
- Difficulty regulating emotions and managing stress.
- Anxiety, boredom, or emotional distress.
- Co-occurring OCD or other mental health conditions.

Treatment

- Cognitive Behavioral Therapy (CBT), particularly Habit Reversal Training (HRT).
- Awareness training and alternative coping strategies.
- Medication may be considered by a physician when appropriate.

Excoriation (Skin-Picking) Disorder

Diagnostic Criteria

F42.4

A. Recurrent skin picking resulting in skin lesions.

B. Repeated attempts to decrease or stop skin picking.

C. The skin picking causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

D. The skin picking is not attributable to the physiological effects of a substance (e.g., cocaine) or another medical condition (e.g., scabies).

E. The skin picking is not better explained by symptoms of another mental disorder (e.g., delusions or tactile hallucinations in a psychotic disorder, attempts to improve a perceived defect or flaw in appearance in body dysmorphic disorder, stereotypies in stereotypic movement disorder, or intention to harm oneself in nonsuicidal self-injury).

Excoriation (Skin-Picking Disorder)



Statistics

- Affects approximately 1–2% of the population.
- Often begins during adolescence.

Causes/Risk Factors

- Genetic predisposition and family history.
- Stress, anxiety, or emotional distress.
- Difficulty regulating emotions.
- Co-occurring OCD or body-focused repetitive behaviors.

Treatment

- Cognitive Behavioral Therapy (CBT), particularly Habit Reversal Training (HRT).
- Stimulus control techniques to reduce skin-picking triggers.
- Medication may be considered by a physician when appropriate.

Substance/Medication-Induced Obsessive-Compulsive and Related Disorder

Diagnostic Criteria

A. Obsessions, compulsions, skin picking, hair pulling, other body-focused repetittive behaviors, or other symptoms characteristic of the obsessive-compulsive and related disorders predominate in the clinical picture.

B. There is evidence from the history, physical examination, or laboratory findings of both (1) and (2):

1. The symptoms in Criterion A developed during or soon after substance intoxication or withdrawal or after exposure to or withdrawal from a medication.
2. The involved substance/medication is capable of producing the symptoms in Criterion A.

C. The disturbance is not better explained by an obsessive-compulsive and related disorder that is not substance/medication-induced. Such evidence of an independent obsessive-compulsive and related disorder could include the following:

The symptoms precede the onset of the substance/medication use; the symptoms persist for a substantial period of time (e.g., about 1 month) after the cessation of acute withdrawal or severe intoxication; or there is other evidence suggesting the existence of an independent non-substance/medication-induced obsessive-compulsive and related disorder (e.g., a history of recurrent non-substance/medication-related episodes).

D. The disorder does not occur exclusively during the course of a delirium.

E. The disturbance causes clinically significant distress or impairment in social, occupational, or other importan areas of functioning.

Substance/Medication-Induced Obsessive-Compulsive and Related Disorder

Note:
This diagnosis should be made in addition to a diagnosis of substance intoxication or substance withdrawal only when the symptoms in Criterion A predominate in the clinical picture and are sufficiently severe to warrant clinical attention.

Coding note:

The ICD-10-CM codes for the [specific substance/medication]-induced obsessive-compulsive and related disorders are indicated in the table. Note that the ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance.

In any case, an additional separate diagnosis of a substance use disorder is not given. If a mild substance use disorder is comorbid with the substance-induced obsessive-compulsive and related disorder, the 4th position character is "1," and the clinician should record "mild [substance] use disorder" before the substance-induced obsessive-compulsive and related disorder (e.g., "mild cocaine use disorder with cocaine-induced obsessive-compulsive and related disorder"). If a moderate or severe substance use disorder is comorbid with the substance-induced obsessive-compulsive and related disorder, the 4th position character is "2," and the clinician should record "moderate [substance] use disorder" or "severe [substance] use disorder," depending on the severity of the comorbid substance use disorder. If there is no comorbid substance use disorder (e.g., after a one-time heavy use of the substance), then the 4th position character is "9," and the clinician should record only the substance-induced obsessive-compulsive and related disorder.

ICD-10-CM Codes

	With mild use disorder	With moderate or severe use disorder	Without use disorder
Amphetamine-type substance (or other stimulant)	F15.188	F15.288	F15.988
Cocaine	F14.188	F14.288	F14.988
Other (or unknown) substance	F19.188	F19.288	F19.988

Specify (see **Table 1** in the chapter "Substance-Related and Addictive Disorders," which indicates whether "with onset during intoxication" and/or "with onset during withdrawal" applies to a given substance class; or specify "with onset after medication use"):

With onset during intoxication:	If criteria are met for intoxication with the substance and the symptoms develop during intoxication.
With onset during withdrawal:	If criteria are met for withdrawal from the substance and the symptoms develop during, or shortly after, withdrawal.
With onset after medication use:	If symptoms developed at initiation of medication, with a change in use of medication, or during withdrawal of medication.

Obsessive-Compulsive and Related Disorder Due to Another Medical Condition

Diagnostic Criteria | F06.8

A. Obsessions, compulsions, preoccupations with appearance, hoarding, skin picking, hair pulling, other body-focused repetitive behaviors, or other symptoms characteristic of obsessive-compulsive and related disorder predominate in the clinical picture.

B. There is evidence from the history, physical examination, or laboratory findings that the disturbance is the direct pathophysiological consequence of another medical condition.

C. The disturbance is not better explained by another mental disorder.

D. The disturbance does not occur exclusively during the course of a delirium.

E. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Specify if:

With obsessive-compulsive disorder-like symptoms:
If obsessive-compulsive disorder–like symptoms predominate in the clinical presentation.

With appearance preoccupations:
If preoccupation with perceived appearance defects or flaws predominates in the clinical presenttation.

With hoarding symptoms:
If hoarding predominates in the clinical presentation.

With hair-pulling symptoms:
If hair pulling predominates in the clinical presentation.

With skin-picking symptoms:
If skin picking predominates in the clinical presentation.

Coding note:

Include the name of the other medical condition in the name of the mental disorder (e.g., F06.8 obsessive-compulsive and related disorder due to cerebral infarction). The other medical condition should be coded and listed separately immediately before the obsessive-compulsive and related disorder due to the medical condition (e.g., I69.398 cerebral infarction; F06.8 obsessive-compulsive and related disorder due to cerebral infarction).

Other Specified Obsessive-Compulsive and Related Disorder

Other Specified | F42.8

This category applies to presentations in which symptoms characteristic of an obsessive-compulsive and related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the obsessive-compulsive and related disorders diagnostic class. The clinician records the specific reason (e.g., "obsessional jealousy").

1. **Body dysmorphic-like disorder with actual flaws:** *Similar to* body dysmorphic disorder except that the defects or flaws in physical appearance are clearly observable by others (i.e., more noticeable than "slight"). The preoccupation with these flaws is clearly excessive and causes significant distress or impairment.

2. **Body dysmorphic-like disorder without repetitive behaviors:** Presentations that meet body dysmorphic disorder criteria except that the individual has never performed repetitive behaviors or mental acts in response to the appearance concerns.

3. **Other body-focused repetitive behavior disorder:** Presentations involving recurrent body-focused repetitive behaviors other than hair pulling and skin picking (e.g., nail biting, lip biting, cheek chewing) that are accompanied by repeated attempts to decrease or stop the behaviors and that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

4. **Obsessional jealousy:** Characterized by non-delusional preoccupation with a partner's perceived infidelity. The preoccupations may lead to repetitiive behaviors or mental acts in response to the infidelity concerns; they cause clinically significant distress or impairment; and they are not better explained by another mental disorder such as delusional disorder, jealous type, or paranoid personality disorder.

5. **Olfactory reference disorder (olfactory reference syndrome):** Persistent preoccupation with the belief that one emits a foul or offensive body odor that is unnoticeable or only slightly noticeable to others; often involves repetitive and excessive behaviors such as checking for body odor, excessive showering, or seeking reassurance, as well as attempts to camouflage the perceived odor. Causes clinically significant distress or impairment. Known as *jikoshu-kyofu* in traditional Japanese psychiatry.

6. **Shubo-kyofu:** A variant of *taijin kyofusho* that is similar to body dysmorphic disorder and involves excessive fear of having a bodily deformity.

7. **Koro:** Related to *dhat syndrome*, an episode of sudden and intense anxiety that the penis in males (or the vulva and nipples in females) will recede into the body, possibly leading to death.

Unspecified Obsessive-Compulsive and Related Disorder

F42.9

This category applies to presentations in which symptoms characteristic of an obsessive-compulsive and related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the obsessive-compulsive and related disorders diagnostic class. The unspecified obsessive-compulsive and related disorder category is used in situations in which the clinician chooses not to specify the reason that the criteria are not met for a specific obsessive-compulsive and related disorder and includes presentations in which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).

CASE: WHAT'S GOING ON?

Alex, a college student, has increasingly avoided classroom presentations.

Before presentations, Alex thinks:

> *"Everyone will notice that I'm shaking. They'll think I'm incompetent."*

Alex rehearses every sentence repeatedly and sometimes skips class.



DISCUSS:

1. What is Alex afraid of?
2. What behaviors maintain the problem?
3. Which disorder best fits the pattern?
4. What psychological treatment principle could be relevant?



NAME THE PATTERN

CASE A	“I worry about almost everything—even when nothing specific is wrong.”	→ ?
CASE B	“My heart suddenly races. I think I’m dying, and now I’m afraid it will happen again.”	→ ?
CASE C	“I know the thought is unreasonable, but I cannot stop checking.”	→ ?
CASE D	“I keep seeing the accident in my mind and avoid the road where it happened.”	→ ?
CASE E	“I know the spider is unlikely to hurt me, but I cannot get near it.”	→ ?

COMPLETE THE SENTENCE

1. Anxiety is generally _____ oriented, while fear is more _____ oriented.

2. Avoidance can maintain anxiety because it produces _____-term relief.

3. In OCD, an intrusive thought is an _____, while the repetitive response is a _____.

4. The four major PTSD symptom clusters include intrusion, avoidance, negative changes, and _____.

5. ERP stands for Exposure and _____ Prevention.



WHAT SHOULD YOU REMEMBER?

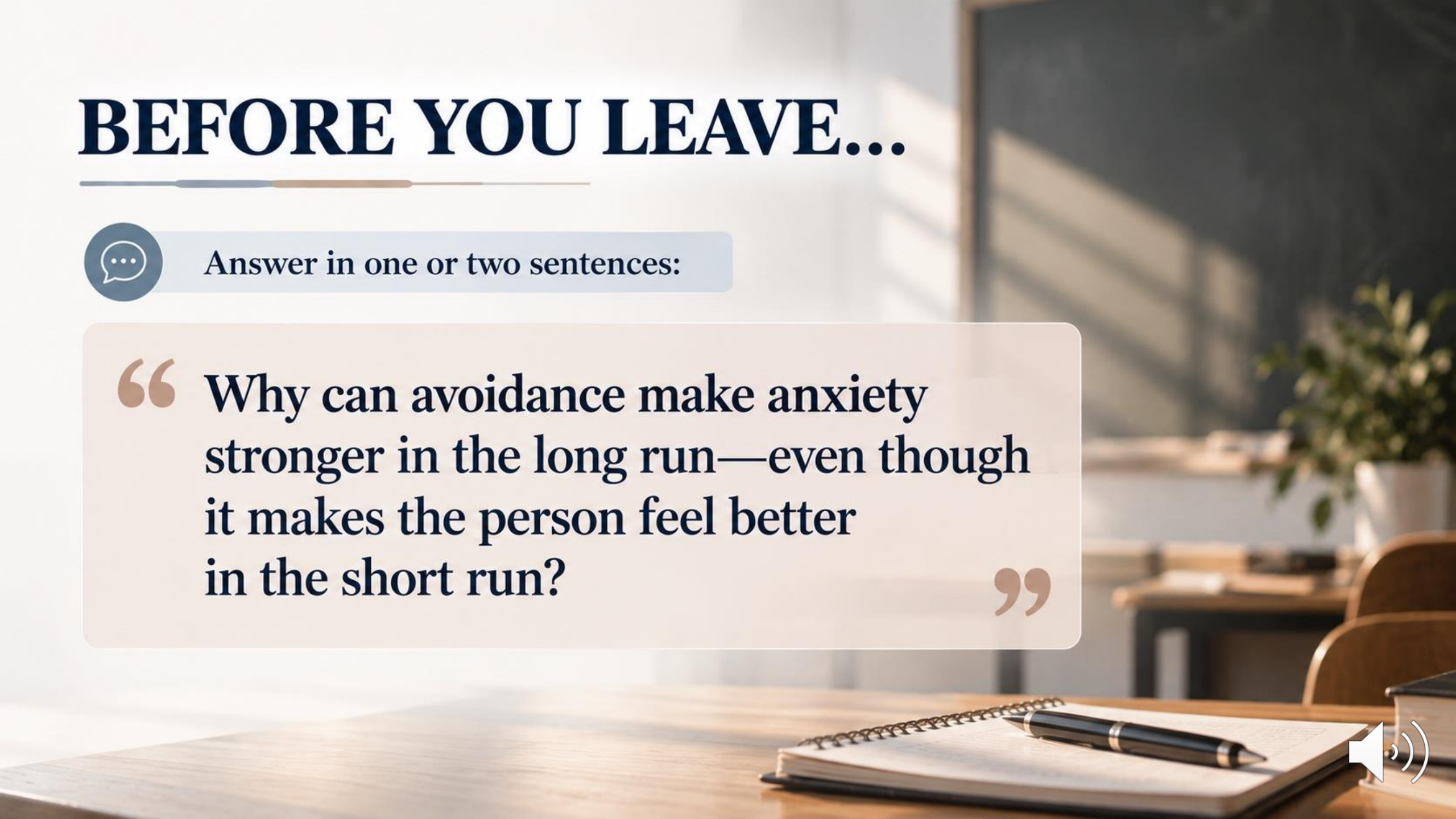
1. Anxiety is **not automatically pathological.**
2. Many anxiety problems are maintained by **avoidance and safety behaviors.**
3. Different disorders differ in **what is feared and how the person responds.**
4. PTSD involves interconnected changes in **intrusion, avoidance, cognition/mood, and arousal/reactivity.**
5. OCD is characterized by **obsessions and/or compulsions** that become clinically significant.
6. Effective treatment often involves helping people **approach rather than continually avoid** feared experiences.



BEFORE YOU LEAVE...

Answer in one or two sentences:

> Why can avoidance make anxiety stronger in the long run—even though it makes the person feel better in the short run?



HOMEWORK: CASE FORMULATION

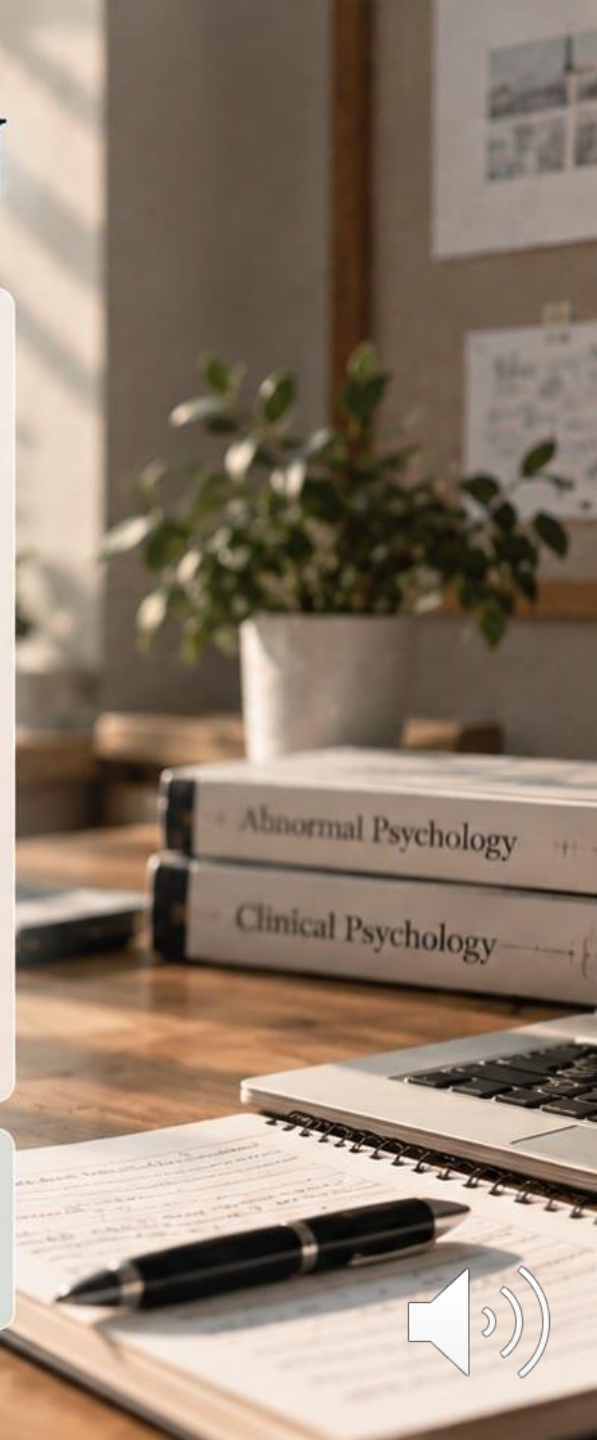
Choose ONE:

- Generalized Anxiety Disorder
- Panic Disorder
- Specific Phobia
- Social Anxiety Disorder
- PTSD
- OCD

Prepare a 1-page mini case formulation containing:

1. Presenting problem
2. Core fear / concern
3. Maintaining behavior
4. Possible biological, psychological, and environmental factors
5. Appropriate evidence-based intervention

Reminder:
Do not diagnose a real person based on this activity.



REFERENCES

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